Mental Health Manual (1)-1

Health records and information technology (Kenya Medical Training College)

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MODULE 20: MENTAL HEALTH

Code: MEH 408

Hours: 80

Credit: 8

Module Competence

This module is designed to enable the learner identify mental health problems and initiate prevention programmes at community level.

Module Outcome

By the end of the module, the learner should;

1. Explain the concepts of mental health.
2. Explain mental disorders.
3. Incorporate mental health primary health care activities.
4. Mobilize community resources to provide and coordinate mental health programs.
5. Explain the myths and misconceptions about mental disorders.

Module Units

Unit Name	Hours	
	Theory	**Practicum**
Introduction to concepts in mental health	10	00
Mental disorders	20	00
Integration of mental health in PHC	16	04
Promotion and prevention programmes	14	06
Myths and misconceptions about mental disorders	10	00

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UNIT 1: INTRODUCTION TO CONCEPTS IN MENTAL HEALTH

What is Mental Health?

It is a state of well-being in which the individual realizes his/her potentials/abilities, can cope with normal stresses of life, can work productively and fruitfully and is able to make a contribution to his/ her community.

Promotion of good Mental Health

Mental health promotion should be availed to individuals, families, communities and the society. This will be achieved through Primary Health Care spearheaded by primary health care team at the district level in close collaboration with other government sectors, NGO's, CBO's, FBO's and private sector.

Good mental health of individuals, families the communities and the society contribute enormously towards investment and development of social capital which is the most important determinant of our health. This endeavor of promotion of good mental health should be encouraged throughout the human development life cycle, right from pregnancy to old age. This will enable the nation to realize its goals and objectives of the Kenya vision 2030.

Prevention of mental disorders and substance abuse and substance dependence

It is evident that the problem of substance abuse and dependence is on the increase especially amongst the youth. This is largely due to the inability to effectively deal with stress. There is need therefore to increase the knowledge, skills and change of attitude in order to deal with the psychological consequences of stress among vulnerable groups.

In order to be effective in the prevention of mental disorders, substance abuse and substance dependence, it is imperative that close inter-sectoral collaboration needs to be developed amongst sectors and partners.

Treatment of mental disorders, substance abuse and substance dependence

In line with Kenya's Health Police Framework and Health Sector Reforms, it is envisaged that health services delivery system will be decentralized and integrated throughout the health care delivery system. Therefore, mental health services are no exception. Therefore, treatment of mental disorders and substance abuse and substance dependence will be decentralized and integrated up to the community level. This is possible because all hospitals

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are gazetted to be mental hospitals for admission and treatment of people suffering from mental disorders in accordance with Sec. 9 of sub-section 2 of the Mental Health Act 1989; Cap 248 of the Laws of Kenya. Mental health services should be provided at different levels of care.

The Burden of Mental illness

There is a report that 450 million people alive today suffer from mental disorders and psychosocial problems such as alcohol and substance dependence and the majority of them suffer silently. One person in every four will be affected by a mental disorder at some stage of life. Major depression is the leading cause of disability and ranks fourth in the ten leading causes of the global burden of disease and is projected to be ranked second within the next 20 years. At least 10 million people attempt suicide every year and one million of them including many who are very young do kill themselves.

The World Health Report 2003 by WHO states that mental, neurological and substance abuse disorders is a major contributor of the overall Global Burden of Disease (GBD). Globally, 13% of overall disability – adjusted life years (DALYs) and 33% of overall years lived with disability (YLDs) are attributable to mental disorders. A large proportion of individuals do not receive any health care for their condition because the mental health infrastructure and services in most countries are grossly insufficient and very insufficient funding and implementation of programs largely due to prevalent
stigma and discrimination of patients requiring the services.

Rehabilitation of mentally sick persons and substance dependent persons

The psychosocial, occupational and behavioral rehabilitation will be offered at primary, secondary and tertiary levels. However, there would be need for continuing rehabilitation to the family and every community should be encouraged to establish recreational and therapeutic facilities with a view to socially integrates the person to the family and the society at large.

Brief History of Mental health

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Primitive beliefs regarding mental disturbances took several views. Some people thought that an individual with mental illness had been dispossessed of his or her soul and that the only way to wellness would be achieved was if the soul returned.

Others believed that evil spirits or supernatural or magical powers had entered the body. The "care" for these individuals involved a ritualistic exorcism to purge the body of these unwanted forces. This reason consisted of brutal beatings; starvation, or other torturous means.

Still others considered that the mentally ill individuals may have broken a taboo or sinned against another individual or God, for which ritualistic purification was required or various types of retribution were demanded. The correlation of mental illness to demonology or witchcraft led to some mentally ill individuals being burned at the stake.

The position of these ancient beliefs evolved with increasing knowledge about mental illness, as well as changes in cultural, religious, and socio-political attitudes. The work of Hippocrates, about 400 BC, began the movement away from belief in the supernatural powers. Hippocrates associated insanity and mental illness with an irregularity in the interaction of the four (4) body fluids in describing about personalities in psychology.

Disequilibrium among these four humors was thought to cause mental illness and it was often treated by inducing vomiting and diarrhea with potent cathartic drugs.

The Middle Ages

During the Middle Ages- AD 500 to 1500, the association of mental illness with witchcraft and supernatural phenomena continued to prevail in the European community. During this period, many severely mentally ill people were sent out to the sea on sailing boats with little guidance and in search of their lost rationality. This operation is credited with engendering the expression "Ship of Fools".

In the same period, in the Middle East- Islamic countries, however, a change in attitude began to occur, from one of mental illness as a result of witchcraft or supernatural to the idea that these individuals were actually ill. This notion give rise to the establishment of "Special Units" for the mentally ill within the general hospitals, as well as institutions specifically designed to house the insane. They can likely be considered the first **Asylums** for the mentally ill.

Colonial Americans tended to reflect the attitudes of the European community from which they had immigrated. Particularly in the New England area, individuals were punished for behavior attributed to witchcraft. In the 16th and 17th centuries, institutions for the mentally

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ill were not available in America and care of these individuals became a family responsibility. Those without a family or other resources became the responsibility of the communities in which they lived and were incarcerated in places where they could do no harm to themselves or others.

The first hospital in America to admit mentally ill patients was established in Philadelphia in the middle of the 18th century. Benjamin Rush, often called the father of American Psychiatry, was a physician at the hospital. He initiated the provision of humanistic treatment and care of the mentally ill. Although he included kindness, exercise and socialization, he also employed hasher methods such as blood-letting; purging, various types of physical restraints and extremes of temperatures, reflecting the medical therapies of that era.

The 19th century brought the establishment of a system of state asylums, largely the result of the work of Dorothea Dix, a former New England school teacher, who lobbied tirelessly on behalf of the mentally ill population. She was unfaltering in her belief that mental illness was curable and that state hospitals should provide humanistic therapeutic care. This system of hospital care for the mentally ill grew, but the mentally ill population grew faster. The institution became over-crowded and under-staffed and conditions deteriorated. The therapeutic care reverted to "custodian care". These state hospitals provided the largest resources for the mentally ill until the initiation of the community health movement of the 1960's.

In 1949, the colonial parliament enacted a law, "the Mental Treatment Act", based on a prototype borrowed from Britain. In 1959, the "Mental Treatment Act" was replaced with a more humane Mental health Act, which placed emphasis on community mental health services, repeating the spirit of the new Act in Britain.

The current Mental Health Act became operational in 1959, and for the first time provides for the voluntary treatment of people with mental illness. It also created a regulatory body to oversee its implementation.

As a result of the 1989 **Mental Health Act,** over half of all government general hospitals opened their doors for psychiatric patients.

Concepts of Mental Health

Mental health does not mean absence of mental illness but a sense of wellbeing felt. Mental health and physical health are interrelated and interdependent. Health

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Definitions

Health: "A state of complete physical, mental and social well-being and not merely the absence of disease or infirmity" (WHO 1948).

Mental health: The capacity of an individual to form harmonious relationships with others and to participate in or contribute constructively to changes in the social environment. (WHO)

Mental illness/disorder: "Maladaptive responses to stressors from the internal or external environment, evidenced by thoughts, feelings, and behaviors that are incongruent with the local and cultural norms, and interfere with the individual's social, occupational, and/or physical functioning" (Townsend, 2009).

Consequences of Mental illnesses

- Poverty
- Poor/inhumane living conditions
- Human rights violation
- Marginalization & vulnerability
- Higher rates of mortality & disability
- Physical abuse
- Sexual abuse
- Neglect
- Loss of civil and political rights

Determinants of Mental Health

- Individual factors e.g., ability to manage one's thoughts, emotions, behaviors and interactions with others.
- External factors e.g.
- Social
- Cultural
- Economic
- Political and
- Environmental

Characteristics of a Mentally healthy person

- Free from internal conflicts (not at war with self)

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- Accepts self and others (well adjusted)
- Ability to cope or tolerate stress.
- Ability to form close and lasting relationships
- Uses sound judgment to make decisions
- Accepts responsibility for actions
- Optimistic in life
- Has good control over own behavior
- Function effectively & independently (productive)
- Able to perceive imagined circumstances from reality
- Able to develop potential and talents to fullest extent
- Able to solve problems
- Can delay immediate gratification
- Searches for identity
- Has strong sense of self esteem
- Recognizes limitations (abilities and deficiencies)

Characteristics of a Mentally ill person

- Feelings of inadequacy (Poor self-concept)
- Inability to cope (Maladaptive behavior)
- Inability to establish a meaningful relationship
- Displays poor judgment
- Irresponsibility or inability to accept responsibility for actions
- Pessimistic in life
- Does not recognize limitations (abilities and deficiencies
- Exhibits dependency needs because of feelings of inadequacy
- Inability to perceive reality (Lives in fantasy)
- Does not recognize potential and talents due to a poor self-concept (Not strive for actualization)
- Avoids problems rather than handling them or attempting to solve them.
- Desires or demands immediate gratification

Point to summarize:

Positive mental health includes emotion, cognition, and social functioning and coherence. (WHO: 2009).

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Whereby an individual has ability to maintain: -

- An even temper
- An alert intelligence
- A socially considerate behavior
- A happy disposition

The mentally healthy person: -

- Accepts the self
- Is self-reliant, and
- Self- confident.

Aetiological factors of Mental Illnesses

The causes of mental illness can be classified into three broad categories:

- Predisposing factors
- Precipitating factors
- Perpetuating factors

Predisposing Factors

Determines the likelihood of one getting a mental illness. Usually, they are adverse experiences one undergoes in early life.

Precipitating Factors

Events that take place shortly before the onset of a disorder. E.g., drug abuse, loss of a job.

Perpetuating Factors

Once the disorder has been triggered, these factors do prolong the course of the disease. They may appear long after the original predisposing factors have been treated. Examples include secondary demoralization and withdrawal from social activities.

Biological factors

- Genetic factors: inheritance of faulty genes and chromosomal abnormalities e.g., down syndrome, Alzheimer's disease.
- Physical illness leading to loss of mental capacity e.g., acute malaria
- Emotional factors: anxiety and neurotic personality traits cause people to develop psychosomatic disorder Infection, Disease and Toxins

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- Brain defects or injury: Defects in or injury to certain areas of the brain have also been linked to some mental illnesses.
- Others such as prenatal damage(autism), nutrition related, substance abuse etc.

Psychological Factors

Psychological factors that may contribute to mental illness include:

- Severe psychological trauma suffered as a child, such as emotional, physical, or sexual abuse
- An important early loss, such as the loss of a parent
- Neglect
- Poor ability to relate to others

Socio cultural factors

- Social influence- maladaptive behavior
- Low socio-economic class.
- Social role- a soldier kills then later develops guilty conscious leading to mental illness
- Violence
- Unemployment
- Broken homes

Factors that Influence Attitudes towards Mental Health and Mental Illness in the community.

Culture

The way people think, behave or feel is shaped by their culture. Culture also determines the features of insanity, for example, who is labelled as insane and under what circumstances.

What is considered insane in one culture may be considered perfectly normal in another. Culture also gives guidelines on the nature of treatment and the identity of the helper.

Education

The level of education also influences attitudes towards mental health and mental illness.

An educated person has a better understanding of health and mental illness, thus making their attitude more positive.

Health Beliefs

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These will determine whether the individual's attitude is positive or negative. It will depend on how the patient explains the illness to themselves, that is, whether they believe in germ theory, evil spirits or an imbalance of some kind.

Religion

A patient's reaction to mental illness will often depend on whether or not the patient believes in God or a particular religion, for example, some religions believe that ill health is caused by evil spirits. Usually, religion encourages the followers to be empathetic to sick people.

2.0 MENTAL HEALTH DISORDERS 20 HOURS

By the end of the unit learners will be able to:-

- Differentiate between the two major systems of classifying mental disorders
- Classify mental disorders according to each classification system

2.1 classification of mental health disorders

1. **International classification of diseases (ICD)-**by world health organization.

    - Currently revised to 11th edition.

2. **Diagnostic Statistical Manual of mental disorders (DSM)-**by American Psychiatric Association.

    - Currently in its 5th edition.

1.1.1 Diagnostic Statistical Manual of mental disorders (DSM)

- Neurodevelopmental Disorders.
- Schizophrenia Spectrum and Other Psychotic Disorders.
- Bipolar and Related Disorders.
- Depressive Disorders.
- Anxiety Disorders.
- Obsessive-Compulsive and Related Disorders.
- Trauma- and Stressor-Related Disorders.
- Dissociative Disorders
- Somatic Symptom and Related Disorders.
- Feeding and Eating Disorders.
- Elimination Disorders.

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- Sleep-Wake Disorders.
- Sexual Dysfunctions.
- Gender Dysphoria.
- Disruptive, Impulse-Control, and Conduct Disorders.
- Substance-Related and Addictive Disorders.
- Neurocognitive Disorders.
- Personality Disorders.
- Paraphilic Disorders.
- Other Mental Disorders.
- Medication-Induced Movement Disorders and Other Adverse Effects of Medication.
- Other Conditions That May Be a Focus of Clinical attention

International classification of diseases (ICD_{11})

- 05 A: Neurodevelopmental disorders
- 05 B: Schizophrenia Spectrum and Primary Psychotic Disorders
- 05 C: Bipolar and Related Disorders
- 05 D: Depressive Disorders
- 05 E: Anxiety and Fear Related Disorders
- 05 F: Disorders Specifically Associated with stress
- 05 G: Dissociative Disorders
- 05 H: Bodily Distress Disorders and Psychological and Behavioral Factors Associated with Diseases Classified Elsewhere
- 05 I: Obsessive Compulsive and Related Disorders
- 05 J: Feeding and Eating Disorders
- 05 K: Elimination Disorders
- 05 L: Sleep Disorders
- 05 M: Sexual Dysfunctions and Compulsive Sexual Behavior Disorder
- 05 N: Acute Substance Intoxication

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- 05 O: Harmful Use of Substances
- 05 P: Substance Dependence
- 05 Q: Substance Withdrawal Syndrome
- 05 R: Substance-induced mental disorders
- 05 S: Behavioral addictions
- 05 T: Disruptive behavior and dissocial disorders
- 05 U: Disorders of Personality

2.2.0Anxiety Disorders

2.2.1 Introduction

2.2.2 Learning Outcomes

2.2.3 Purpose

The purpose of this unit is to equip the students with knowledge, skills and attitudes to enable them create awareness, identify, demystify and assist in management, effective referral and follow up of anxiety disorders in the community.

2.2.4 Definition of Anxiety Disorder

Anxiety disorders include frequently experiencing intense excessive and persistent worry and fear about everyday situations.

2.2.5 Types of anxiety disorders

Four types of anxiety disorders include:

a. **Generalized anxiety disorder**
b. **Phobic disorders**
c. **Obsessive-compulsive disorder (OCD),** and
d. **Post-traumatic stress disorder (PTSD)**.

Generalized Anxiety Disorder

Generalized Anxiety Disorder is severe ongoing worry that interferes with daily activities. It is characterized by persistent and excessive worry about a number of things like anticipated disaster, concern about money, health, family and work. It is difficult to control the worry.

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It is possible to develop generalized anxiety disorder as a child or an adult. It has symptoms that are similar to panic disorder, obsessive-compulsive disorder and other types of anxiety, but they are all different conditions.

Living with generalized anxiety disorder can be a long-term challenge. In many cases, it occurs along with other anxiety or mood disorders. In most cases, generalized anxiety disorder improves with psychotherapy or medication. Making lifestyle changes, learning coping skills and using relaxation techniques also can help.

Symptoms of Generalized Anxiety Disorder

Generalized anxiety disorder symptoms can vary. They may include:

i. Persistent worrying or anxiety about a number of areas that are out of proportion to the impact of the events.
ii. Overthinking plans and solutions to all possible worst-case outcomes
iii. Perceiving situations and events as threatening, even when they are not.
iv. Difficulty handling uncertainty.
v. Indecisiveness and fear of making the wrong decision.
vi. Inability to set aside or let go of a worry
vii. Inability to relax, feeling restless, and feeling keyed up or on edge
viii. Difficulty concentrating, or the feeling that your mind "goes blank"

Physical signs and symptoms may include:

- Fatigue
- Trouble sleeping
- Muscle tension or muscle aches
- Trembling, feeling twitchy
- Nervousness or being easily startled
- Sweating
- Nausea, diarrhea or irritable bowel syndrome
- Irritability

Note:
There may be times when your worries do not completely consume you, but you still feel anxious even when there is no apparent reason. For example, you may feel intense worry about your safety or that of your loved ones, or you may have a general sense that something bad is about to happen.

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Your anxiety, worry or physical symptoms cause you significant distress in social, work or other areas of your life. Worries can shift from one concern to another and may change with time and age.

Symptoms of Anxiety Disorder in children and teenagers

Children and teenagers may have similar worries to adults, but also may have excessive worries about:

- Performance at school or sporting events
- Family members' safety
- Being on time (punctuality)
- Earthquakes, nuclear war or other catastrophic events

A child or teen with excessive worry may:

- Feel overly anxious to fit in
- Be a perfectionist
- Redo tasks because they aren't perfect the first time
- Spend excessive time doing homework
- Lack confidence
- Strive for approval
- Require a lot of reassurance about performance
- Have frequent stomachaches or other physical complaints
- Avoid going to school or avoid social situations

Note:

Some anxiety is normal but one should seek medical attention when:

- One feel like worrying too much, and it's interfering with work, relationships or other parts of life
- One feel like depressed or irritable, have trouble with drinking or drugs, or have other mental health concerns along with anxiety
- One feels like have suicidal thoughts or behaviors — seek treatment immediately

2.2.6 Causes of Anxiety Disorder

As with many mental health conditions, the cause of generalized anxiety disorder likely arises from a complex interaction of biological and environmental factors, which may include:

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- Differences in brain chemistry and function
- Genetics
- Differences in the way threats are perceived
- Development and personality

2.2.7 Risk factors to Anxiety Disorder

The following factors may increase the risk of developing generalized anxiety disorder:

1. **Personality** - A person whose temperament is timid or negative or who avoids anything dangerous may be more prone to generalized anxiety disorder than others are.

2. **Genetics**- Generalized anxiety disorder may run in families.

3. **Experiences**. People with generalized anxiety disorder may have a history of significant life changes, traumatic or negative experiences during childhood, or a recent traumatic or negative event. Chronic medical illnesses or other mental health disorders may increase risk.

Complications of Anxiety Disorder

Having generalized anxiety disorder can be disabling. It can:

- Impair your ability to perform tasks quickly and efficiently because you have trouble concentrating
- Take your time and focus from other activities
- Sap your energy
- Increase your risk of depression

Generalized anxiety disorder can also lead to or worsen other physical health conditions, such as:

- Digestive or bowel problems, such as irritable bowel syndrome or ulcers
- Headaches and migraines
- Chronic pain and illness
- Sleep problems and insomnia
- Heart-health issues

Generalized anxiety disorder often occurs along with other mental health problems, which can make diagnosis and treatment more challenging

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2.2.8 Prevention of Anxiety Disorder

There is no way to predict what will cause someone to develop generalized anxiety disorder, but you can take steps to reduce the impact of symptoms if you experience anxiety:

1. **Get help early**. Anxiety, like many other mental health conditions, can be harder to treat if you wait.
2. **Keep a journal**. Keeping track of your personal life can help you and your mental health professional identify what's causing you stress and what seems to help you feel better.
3. **Prioritize issues in your life**. You can reduce anxiety by carefully managing your time and energy.
4. **Avoid unhealthy substance use**. Alcohol and drug use and even nicotine or caffeine use can cause or worsen anxiety. If you are addicted to any of these substances, quitting can make you anxious. If you cannot quit on your own, see your doctor, find a treatment program, or support group to help you

2.3.0 Cerebral palsy

Cerebral Palsy is a neurological disorder that refers to a group of non-progressive disorders affecting movement, balance and posture. The condition stems from something anomalous happening to the developing brain or from brain damage before birth, during birth or within the first three years of life (and that is why it is called Infantile).

In Cerebral Palsy, signals from the brain to the body are lost or redirected, resulting in difficulties for people with Cerebral Palsy.

2.3.1 Types of Cerebral Palsy

Cerebral Palsy manifests itself in a variety of ways. The three main types of Cerebral Palsy are:

a. **Spastic Cerebral Palsy** (the most common, characterized by muscular stiffness in at least one limb),
b. **Atetoid Cerebral Palsy** (characterized by involuntary movement) and
c. **Ataxic Cerebral Palsy** (Characterized by difficulties in coordinated movement).

However, the limits between one classification and the other are not so clear and there may be mixed forms of Infantile Cerebral Palsy, or other categorization.

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Cerebral Palsy Severity Levels

Cerebral Palsy has a wide range of severity. Some people with Cerebral Palsy may not be blatantly disabled or may seem slightly untidy while others have very limited control of their bodily movements and may have other neurological symptoms, such as eyesight and hearing problems, convulsions, intellectual disability or difficulty in speaking.

Depending On the severity of their condition and their needs, people with Cerebral Palsy can use guardians, sticks, crutches, ambulators or wheelchairs to move, but not all need assistive mobility devices. A bit as happens to all human beings, some have blond hair, other brown or black, others even red: it pays attention to enhance and not discriminate the various differences.

Table 2: Severity Levels of Cerebral Palsy

Mild Cerebral Palsy	**Moderate – Moderate Cerebral Palsy**	**Severe**
- A person with ***mild CP*** might walk a little awkwardly, but might not need any special help. ***CP*** does not get worse over time, though the exact ***symptoms*** can change over a person's lifetime. All people with ***CP*** have problems with movement and posture.	Means a child will need braces, medications, and adaptive technology to accomplish daily activities.	Severe Cerebral Palsy means a child will require a wheelchair and will have significant challenges in accomplishing daily activities.

VARIOUS THERAPIES IN THE MANAGEMENT OF CEREBRAL PALSY

Many people with cerebral palsy undergo:

- Tendon elongations and other orthopedic procedures, such as serial casting, useful in order to maintain the movement of the limbs.
- Others also develop co-morbidity, such as epilepsy and for this reason; they always carry life-saving drugs.

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- To improve daily life activities, those living with cerebral palsy can attend weekly physiotherapy and occupational therapy sessions, to better develop motor skills and increase independence.
- Speech therapy sessions to improve communication skills and language.

Many children with Cerebral Palsy experience physical pain

Cerebral Palsy can cause progressive muscle wear, particularly over time. The pain in children living with this disability for several years can result from abnormal movements of the muscles, typical of Cerebral Palsy.

People with Cerebral Palsy can be independent

From a young age, many people with Cerebral Palsy learn adaptive skills to maximize their adult independence. Although some people with Cerebral Palsy require high levels of assistance, many others can drive or move independently, live in an independent way and sustain themselves economically.

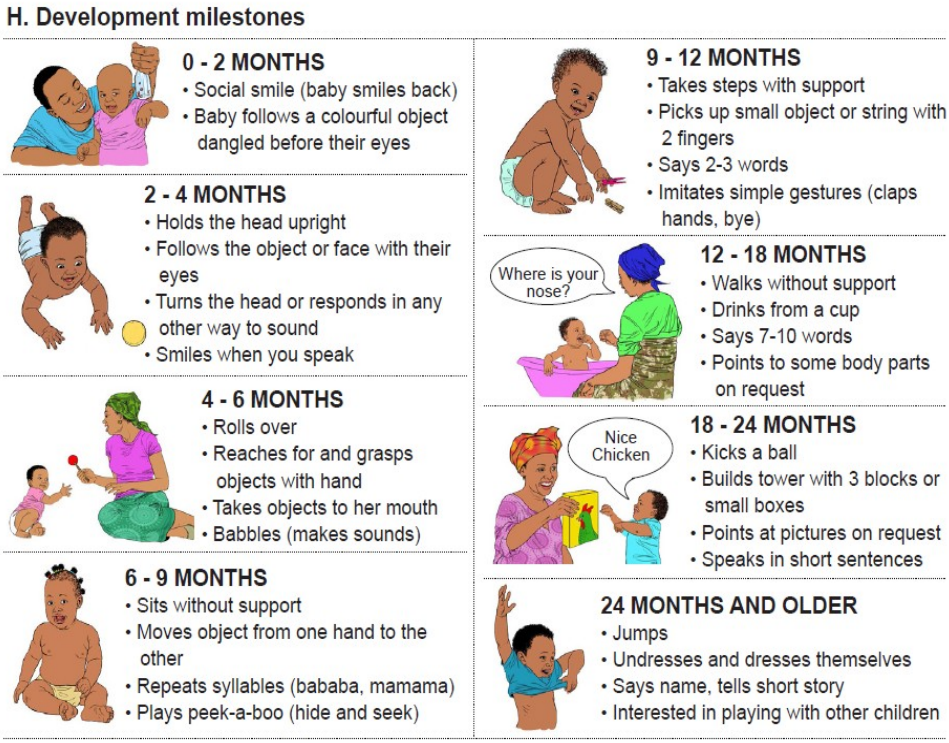

Figure 1: Child Development Milestones Assessment

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*For autism, delays will mainly be noticed with speech, communication, difficulties expressing needs. Vision, Hearing, and other sensory functions should be examined before confirming diagnosis.

2.4.0 Autism

A spectrum of neuropsychiatric disorders characterized by deficits in social interaction and communication, and unusual and repetitive behavior. Some, but not all, people with autism are non-verbal. Autistic disorder is characterized by a **withdrawal of the child into the self and into a fantasy world of his or her own creation.**

The child has markedly abnormal or impaired development in social interaction and communication.

Activities and interests are restricted and may be considered somewhat bizarre.

Etiology

1. Neurological Implications

- Autism thought to be caused by abnormalities in brain structures or functions.
- Abnormalities have been found in the area of the **amygdala**, which is known to help regulate aspects of social and emotional behavior.
- **Elevated levels of serotonin** have also been found in a number of people with autism.

2. Perinatal influences: Women with asthma and allergies recorded during the second trimester had a greater than two-fold elevated risk of having a child affected by autism.

Types of Autism

There are three quite different types of autism spectrum disorders:

i. Autistic disorder/classic autism,
ii. Asperger syndrome and
iii. Pervasive developmental disorder

The most common form of autism spectrum disorder is classic autism

Table 1: Types of Autism

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Types of Autism Spectrum Disorders	Description
Classic autism	Those with this type of spectrum disorder have "significant language delays, social and communication challenges, and unusual behaviors and interests." These people are usually affected by intellectual disabilities as well. This type is considered the most severe form of autism and also the most common.
Asperger syndrome	People with Asperger's may experience the same symptoms as the other types of autism but milder. People with Asperger syndrome have unusual behaviors and interests, in addition to social challenges. They usually don't have problems with language or intellectual disability
Pervasive developmental disorder	This type is typically reserved for those who meet some of the criteria for the other two types, but not all of them. Those affected with PDD-experience milder symptoms or fewer symptoms. Quite typically, they only suffer from social and communication challenges. These people tend to be the highest-functioning autistic types and simply do not fit into any of the other categories or types of autism spectrum disorders.

Diagnostic criteria for Autism Spectrum Disorder

A. Persistent deficits in social communication and social interaction across multiple contexts as manifested by the following, currently or by history.

1. Deficits in social-emotional reciprocity

2. Deficits in nonverbal communicative behaviors used for social interaction e.g.

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- abnormal eye contact and body language or lack of facial expressions
- nonverbal communication.

B. Persistent deficits in social interaction across multiple contexts manifested by the following

1. Deficits in developing, maintaining, and understanding relationships e.g.

- difficulties in sharing imaginative play
- Difficulty in making friends
- absence of interest in peers

C**. Restricted, repetitive patterns of behavior, interests, or activities**, as manifested by at least two of the following

1. Stereotyped or repetitive motor movements, use of objects, or speech e.g.,

- simple motor stereotypies
- lining up toys or flipping objects
- Echolalia
- idiosyncratic phrases

2. Insistence on sameness, inflexible adherence to routines, or ritualized patterns or verbal nonverbal behavior e.g.,

- extreme distress at small changes
- difficulties with transitions
- rigid thinking patterns
- greeting rituals
- need to take same route or eat food every day

3. Highly restricted, fixated interests that are abnormal in intensity or focus e.g.

- strong attachment to or preoccupation with unusual objects
- excessively circumscribed or perseverative interest

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4. Hyper- or hypo reactivity to sensory input or unusual interests in sensory aspects of the environment e.g.

- apparent indifference to pain/temperature
- adverse response to specific sounds or textures
- excessive smelling or touching of objects
- visual fascination with lights or movement

2.5.0 Depression

2.5.1 Purpose of the Unit

The purpose of this unit is to equip healthcare workers with knowledge, skills and attitudes to enable them create awareness, identify, demystify and assist in management, effective referral and follow up of Depression and Bipolar Disorders.

2.5.1 Definition of Depression

Depression is a mood disorder that causes a persistent feeling of sadness and loss of interest. Also called major depressive disorder or clinical depression, it affects feelings, thinking, behavior, and can lead to a variety of emotional and physical problems. There may be trouble doing normal day-to-day activities, and sometimes feeling as if life is not worth living.

Depression affect people of all races and socioeconomic class and can occur at any time. Depression is not same as sadness and grief caused by a challenging life experiences such as death of a loved one. Although depression may occur only once during your life, people typically have multiple episodes and these episodes, symptoms occur most of the day, nearly every day.

The real causes of depression are unknown. However, there are factors thought to contribute to depression.

2.5.2Causes of Depression

It is not known exactly what causes depression. As with many mental disorders, a variety of factors may be involved, such as:

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a. **Biological differences.** People with depression appear to have physical changes in their brains. The significance of these changes is still uncertain, but may eventually help pinpoint causes.
b. **Brain chemistry.** Neurotransmitters are naturally occurring brain chemicals that likely play a role in depression. Recent research indicates that changes in the function and effect of these neurotransmitters and how they interact with neurocircuits involved in maintaining mood stability may play a significant role in depression and its treatment.
c. **Hormones.** Changes in the body's balance of hormones may be involved in causing or triggering depression. Hormone changes can result with pregnancy and during the weeks or months after delivery (postpartum) and from thyroid problems, menopause or a number of other conditions.
d. **Inherited traits.** Depression is more common in people whose blood relatives also have this condition. Researchers are trying to find genes that may be involved in causing depression.

2.5.3 Risk factors to Depression

Depression often begins in the teens, 20s or 30s, but it can happen at any age. More women than men are diagnosed with depression, but this may be due in part because women are more likely to seek treatment.

Factors that seem to increase the risk of developing or triggering depression include:

- Certain personality traits, such as low self-esteem and being too dependent, self-critical or pessimistic
- Traumatic or stressful events, such as physical or sexual abuse, the death or loss of a loved one, a difficult relationship, or financial problems
- Being lesbian, gay, bisexual or transgender, or having variations in the development of genital organs that aren't clearly male or female (intersex) in an unsupportive situation
- History of other mental health disorders, such as anxiety disorder, bipolar disorder, Substance Use Disorders, eating disorders or post-traumatic stress disorder
- Serious or chronic illness, including cancer, stroke, chronic pain or heart disease

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- Certain medications, such as some high blood pressure medications or sleeping pills (talk to your doctor before stopping any medication)

2.5.4 Common signs and symptoms of depression

The signs and symptoms of depression occur at various different levels as follows:

a) Emotional level

- Continuous low mood
- Loss of interest or pleasure
- Hopelessness
- Helplessness
- Worthlessness/low self-esteem

b) Psychological level

- Guilty/negative attitude to self
- Loss of concentration/memory
- Persistent thoughts of death or suicide
- Tearfulness
- Extremely sensitive to criticism
- Negative thoughts about everything

c) Physical level

- Slowing down in activities or agitation
- Tiredness/lack of energy
- Sleep problems (total lack of sleep, difficulty in staying asleep or excessive sleep)
- Disturbed appetite (Increased or decreased
- Loss or increase of weight
- Unkempt

2.5.5 Management of Depression

Medications and psychotherapy are effective for most people with depression. A primary care/ doctor or psychiatrist can prescribe medications to relieve symptoms. However, many people with depression also benefit from seeing a psychiatrist, psychologist or other mental health professional.

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Psychotherapy

Psychotherapy is a general term for treating depression by talking about your condition and related issues with a mental health professional. Psychotherapy is also known as talk therapy or psychological therapy.

Different types of psychotherapy can be effective for depression, such as cognitive behavioral therapy or interpersonal therapy. Your mental health professional may also recommend other types of therapies. Psychotherapy can help you:

- Adjust to a crisis or other current difficulty
- Identify negative beliefs and behaviors and replace them with healthy, positive ones
- Explore relationships and experiences, and develop positive interactions with others
- Find better ways to cope and solve problems
- Identify issues that contribute to your depression and change behaviors that make it worse
- Regain a sense of satisfaction and control in your life and help ease depression symptoms, such as hopelessness and anger
- Learn to set realistic goals for your life
- Develop the ability to tolerate and accept distress using healthier behaviors

2.5.6 Prevention of Depression

There is no sure way to prevent depression. However, these strategies may help:

- **Take steps to control stress,** to increase resilience and boosting of self-esteem.
- **Reach out to family and friends,** especially in times of crisis, that can help through rough spells.
- **Get treatment at the earliest sign of a problem** to help prevent depression from worsening.
- **Consider getting long-term maintenance treatment** to help prevent a relapse of symptoms.

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2.5.7 Types of depression

2.5.7.1 Postpartum Depression

Preamble

Postpartum depression can be moderate to severe depression in a woman after she has given birth. It may occur soon after delivery or up to a year later. However, the most common occurrence is within the first three months after delivery.

2.5.7.2 Causes of Postpartum depression

The exact causes of postpartum depression are unknown. However, changes in hormone levels during and after pregnancy may affect a woman's mood. Many non-hormonal factors may also affect mood during this period. These include:

- Changes in the body during pregnancy and delivery
- Physical health problems during pregnancy and delivery e.g. urinary incontinence
- History of depression
- Changes in work and social relationships
- Limited social support
- Inadequate sleep
- Anxious about your ability to be a good mother

2.5.7.3 Risk factors of Postpartum Depression

- A woman may have higher chances of postpartum depression because she is under 20 years of age.
- Currently abuses alcohol, takes illegal substances, or smokes (these also cause serious health risks for the baby).
- Did not plan the pregnancy, or had mixed feelings about the pregnancy
- Had depression, bipolar or an anxiety disorder before the pregnancy, or with a past pregnancy.
- Had a stressful event during the pregnancy or delivery, including personal illness, death or illness of a loved one, a difficult or emergency delivery, premature delivery, or illness or birth defect in the baby.
- Have a close family member who has had depression or anxiety.
- Have a poor relationship with their significant other or are single.
- Have money or housing problems.

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- Have little support from family, friends, or your spouse or partner.

2.5.8 Management of Postpartum Depression

A new mother who has any symptoms of postpartum depression should contact her doctor or nurse right away to get help.

PSYCHOSOCIAL INTERVENTIONS

i. **Psychoeducation**: Key messages to the person and the carers

- Depression is a very common condition that can happen to anybody.
- The occurrence of depression does not mean that the person is weak or lazy.
- Negative attitudes of others (e.g. "You should be stronger", "Pull yourself together") may be because depression is not a visible condition, unlike a fracture or a wound.
- There is also the misconception that people with depression can easily control their symptoms by sheer willpower.
- People with depression tend to have unrealistically negative opinions about themselves, their life and their future.
- Their current situation may be very difficult, but depression can cause unjustified thoughts of hopelessness and worthlessness.
- These views are likely to improve once the depression improves. Thoughts of self-harm or suicide are common. If they notice these thoughts, they should not act on them, but should tell a trusted person and come

ii. Reduce stress and strengthen social support

- Assess for and try to reduce stressors.
- Reactivate the person's previous social network. Identify prior social activities that, if started again, may potentially provide direct or indirect psychosocial support, e.g. family gatherings, visiting neighbours, and community activities.

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- Joining a support group

iii. Promote functioning in daily activities and community life

Even if it is difficult, encourage the person to try to do as many of the following as possible:

- Try to start again (or continue) activities that were previously pleasurable.
- Try to maintain regular sleeping and waking times. Also try to maintain sleep hygiene
- Try to be as physically active as possible.
- Try to eat regularly despite changes in appetite.
- Try to spend time with trusted friends and family.
- Try to participate in community and other social activities as much as possible.
- Explain to the person and carer that these activities can all help improve mood.

iv. Brief psychological management for depression

Some of the intervention recommended by WHO include;

- Problem management+ (PM+)
- Self help+
- Interpersonal psychotherapy (IPT)
- Healthy Thinking – that describes management of perinatal depression.

2.5.9 Prevention of postpartum depression

Here are some other helpful tips to the new mother:

- Ask your partner, family, and friends for help with the baby's needs and in the home.
- Don't hide your feelings. Talk about them with your partner, family, and friends.
- Don't make any major life changes during pregnancy or right after giving birth.
- Don't try to do too much, or to be perfect.
- Make time to go out, visit friends, or spend time alone with your partner.
- Rest as much as you can. Sleep when the baby is sleeping.

Talk with other mothers or join a support group.

Prevention

Preventive approaches are needed because of the long term effects of maternal Postpartum depression

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- Psychosocial and psychological interventions
    - In-hospital psychological debriefings
    - Antenatal and postnatal classes
    - Home visits
    - Continuity of care and early postpartum follow-ups
    - Professional support provided postnatally

Major Depression Disorder

Major depressive disorder is also called major depression. In this disorder individuals experience episodes of sad mood or anhedonia (loss of interest or pleasure) that last longer than two weeks and significantly disrupt their everyday function. Individuals may also have thoughts of self-harm. Some with this disorder may experience a single episode of severe depression in their lifetimes. For many others, recurrent episodes of such depression will occur throughout their lives.

Causes

- Genetic – depressive disorders can be inherited from one or both parents
- Biological factors- exposure to severe or long lasting stress can change the balance of chemicals in the brain that control brain.
- Environmental factors- families with poor coping abilities, low levels of communication within the family and inconsistent response have a higher chance of developing MDD
- Psychological factors – stress in childhood and adulthood like isolation and lack of support can lead to MDD

Major depressive disorders may also be triggered by:

- Alcohol or drug use
- Certain medical conditions, such as cancer or hypothyroidism
- Particular types of medications including steroids
- Abuse during childhood

Symptoms

- Depressed mood- for children and adolescents this can also be an irritable mood

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- Loss of interest or pleasure which have been present during the same 2week period
- Appetite disturbance – weight loss or gain

The symptoms cause clinically significant distress or impairment in social occupational or other important area.

Management

Psychotherapy

Psychotherapy or talk therapy is common term for treating depression by talking about your condition and related issues with a mental therapy health professional.

- Cognitive behavioral therapy or interpersonal therapy are forms that are also used.
- Electroconvulsive therapy is used in depression to impact the function and effect of neurotransmitter in the brain

Medication

Persistent Depressive disorder

Definition

Persistent depressive disorder is a form of chronic (ongoing depression). Although symptoms of persistent depressive disorder may be less severe than other types of depression, they are longer-lasting in duration.

Symptoms

Symptoms of persistent depressive disorder can cause significant impairment and may include:

- Feelings of hopelessness
- Low self-esteem
- Low energy or fatigue
- Poor appetite or overeating
- Difficulty with concentration and decision-making
- Difficulty falling/staying asleep (insomnia) or sleeping too much (hypersomnia)

Risk factors

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The exact cause of persistent depressive disorder is unknown. As with major depression, it may involve more than one cause, such as:

- **Biological differences.** People with persistent depressive disorder may have physical changes in their brains. The significance of these changes is still uncertain, but they may eventually help pinpoint causes.
- **Brain chemistry.** Exposure to severe or long lasting stress can change the balance of chemicals in the brain that control mood
- **Heredity-** Persistent depressive disorder appears to be more common in people whose blood relatives also have the condition.
- **Life events- experiencing a trauma or hardships may trigger the symptoms in people who have how an inherited susceptibility to develop**

Management

The two primary methods for treating persistent depressive disorder include:

1. Pharmaco-therapy- antidepressant medication and
2. Psychotherapy or talk therapy.

The combination of both longer term treatment methods may prove to be most effective.

Premenstrual Dysphoric Disorder

Definition

Premenstrual dysphoric disorder (PMDD) is a severe form of depression disorder that includes physical and behavioural symptoms that usually resolve with the onset of menstruation.

Risk factors

- Genetic and physiological
- Environmental
- Hormones
- Overweight
- Alcohol or substance abuse

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- Lack of exercise

Symptoms

- Mood change
- Marked irritability or anger
- Increased interpersonal conflicts
- Marked depressed mood
- Sleep disturbance
- Concentration difficulty
- Marked anxiety

Management

- Diet and exercise
- Cognitive Behavioral Therapy
- Medication

2.6.0 Epilepsy

2.6.1 Purpose

The purpose of this unit is to equip the community health personnel with knowledge and skills to enable them create awareness, identify, demystify and assist in management, effective referral and follow up of epilepsy.

2.6.2 Definition of epilepsy

Epilepsy is a chronic brain disorder characterized by repetitive, sudden onset, short-lived seizures occurring more than twice in a year, that may arise from many and varied causes. Other commonly used terms for epilepsy are Convulsions; Seizures; Fits; Attack; Kifafa (Kiswahili).

2.6.3 Common myths and misconceptions associated with epilepsy

Table 6: Myths & Facts about Epilepsy

Myth / Misconception	Fact
Epilepsy is contagious	You cannot get epilepsy from another person
You can swallow your tongue during a seizure	It is physically impossible to swallow your tongue. Do not force something into the mouth of someone having a

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	seizure. Doing so may result in chipped teeth, injured gums, or broken jaws.
Epilepsy is a product of witchcraft	Epilepsy is NOT a product of witchcraft or any superstitious activity. It is a seizure disorder.
Epilepsy is a permanent disorder	Epilepsy is NOT a permanent disorder. Most cases would manifest during childhood but seizures would stop at certain age with treatment.
People with epilepsy look different	People with epilepsy look like anyone else
Epilepsy is a curse	Epilepsy is not a curse but a mental health challenge that can be managed.

2.6.4 Signs and Symptoms of Epilepsy

Epilepsy is caused by abnormal activity in the brain, seizures can affect any process coordinated by the brain and can thus lead to:

- A staring spell
- Temporary confusion
- Involuntary jerking movements of the arms and legs
- Loss of consciousness and awareness.

Symptoms vary depending on the type of seizure. In most cases, a person with epilepsy will tend to have the same type of seizure each time, so the symptoms will be similar from one episode to another.

2.6.5 Classification of epilepsy

Epilepsy can be classified into two major types:

1. Partial epilepsy: Seizures that result from abnormal activity in just one part of the brain with either impairment or retention of consciousness.
2. Generalized epilepsy: Seizures that result from a widespread involvement of large parts of the brain simultaneously leading to loss of consciousness.

2.6.6 Causes of Epilepsy

Some of the common causes of epilepsy include:

1. **Brain Infections**

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- Meningitis
- Cerebral malaria
- HIV-AIDS – opportunistic infections

2. **Trauma**
   - Head injury such as road traffic injuries and assault
   - Birth trauma, especially common in home deliveries where there is prolonged labour.

3. **Lack of oxygen to the brain**
   - Lack of oxygen to the baby's brain at birth (birth asphyxia).

4. **Structural brain problems**
   - Accumulation of fluid in the baby's head
   - Tumours/growths in the head
   - Abscesses - pus accumulation in the brain.

NOTE: There are other unknown causes of epilepsy

5. **Genetic:** Runs in families – team add more content

2.6.7 Triggering Factors of Epilepsy

People with Epilepsy are likely to develop seizures when exposed to certain situations or conditions. These include;

- Non-adherence to treatment
- Sleep deprivation
- Infections,
- Flickering of lights
- Drug and Substance abuse
- Emotional Stress
- Excessive physical exercise.

2.6.8 The Do's and Don'ts during an epileptic seizure

Table 7: Do's & Don'ts

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Dos	DONTs
• Move patient away from danger such as fire, traffic or water ●Take away any objects that could harm the patient ● Loosen tight clothes, remove glasses ● Turn the patient on their left side, recovery position ● Put something soft under the head ● Remain with the patient until he or she regains consciousness ● Allow rest and then resumption of activity the patient was doing, if he feels like it.	● Do NOT try to put anything into the mouth ● Do NOT give anything to drink ● Do NOT try to stop the jerking, or restrain the movements.

Unconscious patients should be placed in this recovery position to minimize the risk of them choking in case they vomit. The recovery position refers to one of a series of variations on a lateral recumbent or three-quarters prone position of the body, into which an unconscious but breathing client can be placed as part of first aid treatment.

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Figure 2: Recovery position (Lying on the left lateral /recovery position side)

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2.6.9 Principles of management of epilepsy

- Community awareness to demystify epilepsy and reduce stigma in order to facilitate early health seeking by the patients
- Teach those close to the patient on what to do in the event of a seizure
- Prompt referral and follow-up of persons with signs of epilepsy for confirmation of diagnosis
- Strict compliance and adherence to the treatment regimen prescribed.

2.6.10 Principles of prevention of epilepsy

The following measures should be considered in the prevention of epilepsy:

- Encourage and support mothers to deliver in health facilities to avoid birth trauma
- Seek treatment promptly to avoid complications of diseases such as meningitis and severe malaria
- Prevention of infectious diseases e.g. malaria, meningitis, HIV/AIDS.
- Encourage and support mothers to ensure their children complete immunization schedule
- prevention of road traffic accidents and other courses of trauma
- Prompt management of fever in children to avoid febrile convulsions.
- Strict adherence to prescribed medication.

2.6.11 Role of Health Care Workers in Management of Epilepsy

Create awareness on epilepsy to the community members so as to:

- Reduce stigmatization of persons
- Demystify the common myths and misconceptions
- Conduct regular home visits for early identification of cases in the community.
- Make referrals of clients with epilepsy and act as link person between patients and the health facilities
- Conduct follow-up visits to client to monitor their drug adherence
- Maintain updated clients' data/records.

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2.7 Schizophrenia and Psychoses

2.7.1 Purpose

The purpose of this unit is to equip healthcare workers with knowledge, skills and attitudes to enable them create awareness, identify, demystify and assist in management, effective referral and follow up of schizophrenia and psychotic cases.

2.7.2 Definition of Schizophrenia

Schizophrenia is a chronic and severe mental disorder characterized by distortions in thinking, perception, emotions, language, sense of self and behaviour. Common experiences include hallucinations (hearing voices or seeing things that are not there) and delusions (fixed, false beliefs).

2.7.3 Causes of Schizophrenia

It's not known what causes schizophrenia, but researchers believe that a combination

- Of genetics, brain chemistry and environment contribute to development of the disorder.
- Problems with certain naturally occurring brain chemicals, including neurotransmitters called dopamine and glutamate, may contribute to schizophrenia.
- Neuroimaging studies show differences in the brain structure and central nervous system of people with schizophrenia indicating that schizophrenia is a brain disease.

2.7.4 Risk factors Associated with Schizophrenia

Although the precise cause of schizophrenia isn't known, certain factors seem to increase the risk of developing or triggering schizophrenia, including:

- Having a family history of schizophrenia
- Some pregnancy and birth complications, such as malnutrition or exposure to toxins or viruses that may impact brain development
- Taking mind-altering (psychoactive or psychotropic) drugs during teen years and young adulthood

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2.7.5 Symptoms of Schizophrenia

Schizophrenia involves a range of problems with thinking (cognition), behavior and emotions. Signs and symptoms may vary, but usually involve delusions, hallucinations or disorganized speech, and reflect an impaired ability to function.

1. Delusions. These are false beliefs that are not based in reality. For example,

One think that they are being harmed or harassed; certain gestures or comments are directed at them;

One feeling that they have exceptional ability or fame or another person is in love with them; or a major catastrophe is about to occur.

2. Hallucinations. These usually involve seeing, feeling things that are not there or hearing things that don't exist.

Hallucinations can be in any of the five senses, but hearing voices is the most common hallucination.

3. Disorganized thinking (speech). Disorganized thinking is inferred from disorganized speech, incoherent or irrelevant speech. Effective communication can be impaired, and answers to questions may be partially or completely unrelated.

Rarely, speech may include putting together meaningless words that can't be understood, sometimes known as word salad.

4. Extremely disorganized or abnormal motor behavior. This may show in a number of ways, from childlike silliness to unpredictable agitation.

Behavior isn't focused on a goal, so it's hard to do tasks. Behavior can include resistance to instructions, inappropriate or bizarre posture, a complete lack of response, or useless and excessive movement.

Abnormal behaviour: disorganized behaviour such as wandering aimlessly, mumbling or laughing to self, strange appearance, self-neglect or appearing unkempt.

5. Negative symptoms. This refers to reduced or lack of ability to function normally.

For example, the person may neglect personal hygiene or appear to lack emotion (doesn't make eye contact, doesn't change facial expressions or speaks in a monotone).

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Also, the person may lose interest in everyday activities, socially withdraw or lack the ability to experience pleasure.

6. Positive symptoms: This refers to symptoms that are excess or added to normal mental functioning. Feelings or behaviors that are usually not present, such as believing that what other people are saying is not true (delusions); hearing, seeing, tasting, feeling, or smelling things that others do not experience (hallucinations); disorganized speech and behavior.

Note:
Also, recreational substance use, such as marijuana, methamphetamines or LSD, can sometimes cause similar signs and symptoms.
Compared with schizophrenia symptoms in adults, teens may be:

- Less likely to have delusions

More likely to have visual hallucinations

Symptoms can vary in type and severity over time, with periods of worsening and remission of symptoms. Some symptoms may always be present. Schizophrenia symptoms typically start in the early to mid-20s. It is uncommon for children to be diagnosed with schizophrenia and rare for those older than age 45.

Symptoms in Special Populations (teenagers)

Schizophrenia symptoms in teenagers are similar to those in adults, but the condition may be more difficult to recognize. This may be in part because some of the early symptoms of schizophrenia in teenagers are common for typical development during teen years, such as:

- Withdrawal from friends and family
- A drop in performance at school
- Trouble sleeping
- Irritability or depressed mood
- Lack of motivation

2.7.6 Prevention of schizophrenia

Prevention of schizophrenia is difficult as there are no reliable markers for the later development of the disorder. There is no sure way to prevent schizophrenia, but sticking with

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the treatment plan can help prevent relapses or worsening of symptoms. In addition, researchers hope that learning more about risk factors for schizophrenia may lead to earlier diagnosis and treatment.

However, there is tentative evidence for the effectiveness of the following measures;

- Avoid drugs that have been associated with development of the disorder, including cannabis, cocaine, and amphetamines
- Cognitive behavioral therapy may reduce the risk of psychoses in those at high risk after a year
- Early treatment for those experiencing symptoms of psychoses for the first time

2.7.7 Management of Schizophrenia

2.7.7.1 Psychosocial interventions

Once psychoses recedes, in addition to continuing on medication, psychological and social (psychosocial) interventions are important. These may include:

- **Individual therapy.** Psychotherapy may help to normalize thought patterns. Also, learning to cope with stress and identify early warning signs of relapse can help people with schizophrenia manage their illness.
- **Social and life skills training.** This focuses on improving communication and social interactions and improving the ability to participate in daily activities.
- **Family therapy.** This provides support and education to families dealing with schizophrenia and educating the public to decrease stigma and discrimination.
- **Vocational rehabilitation and supported employment.** This focuses on helping people with schizophrenia prepare for, find and keep jobs facilitating independent living.
- During crisis periods or times of severe symptoms, refer to the mental health professional
- A case manager or someone on the treatment team can help find resources. With appropriate treatment, most people with schizophrenia can manage their illness.

Note:
Most individuals with schizophrenia require some form of daily living support.
Many communities have programs to help people with schizophrenia get jobs,

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housing, join self-help groups and manage crises situations.

2.7.7.2 Electroconvulsive Therapy

For adults with schizophrenia who do not respond to drug therapy, electroconvulsive therapy (ECT) may be considered.

2.7.7.3 Refer for Medical Management

During crisis periods or times of severe symptoms, refer the patient a mental health professional.

2.7.7.4 Prevention of Schizophrenia

There is no sure way to prevent schizophrenia, but sticking with the treatment plan can help prevent relapses or worsening of symptoms. In addition, researchers hope that learning more about risk factors for schizophrenia may lead to earlier diagnosis and treatment.

2.8 Dementia

2.8.1 Purpose

The purpose of this unit is to equip healthcare workers with knowledge, skills and attitudes to enable them create awareness, identify, demystify and assist in management, effective referral and follow up of Dementia

2.8.2 Definition of dementia

Dementia is a general term for loss of memory, language, problem-solving and other thinking abilities that are severe enough to interfere with daily life. Dementia is not a single disease; it's an overall term — like heart disease — that covers a wide range of specific medical conditions, including Alzheimer's disease.

Disorders grouped under the general term "dementia" are caused by abnormal brain changes. These changes trigger a decline in thinking skills, also known as cognitive abilities, severe enough to impair daily life and independent function. They also affect behavior, feelings and relationships. Dementia is not part of normal ageing. Although it can occur at any age, it's more common in older people.

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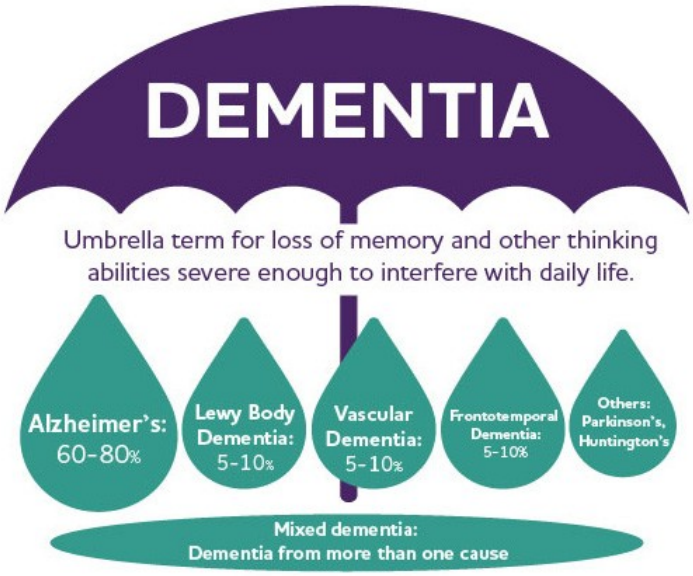

Figure 3:Dementia

2.8.3 Types of dementia

Table 8: Dementia types

Type	Explanation
Alzheimer's disease	Alzheimer's is a degenerative brain disease that is caused by complex brain changes following cell damage. It leads to dementia symptoms that gradually worsen over time. . It accounts for 60-80% of cases.
Vascular dementia	It occurs because of microscopic bleeding and blood vessel blockage in the brain, is the second most common cause of dementia.
Lewy body dementia (LBD)	is the third most common cause of dementia after Alzheimer's disease and vascular dementia, accounting for 5 to 10 percent of cases. It is a type of progressive dementia that leads to a decline in thinking, reasoning and independent function because of abnormal microscopic deposits that damage brain cells over time
Frontotemporal dementia (FTD) or	Refers to a group of disorders caused by progressive nerve cell loss in the brain's frontal lobes (the areas behind your forehead)

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frontotemporal degenerations	or its temporal lobes (the regions behind your ears). The nerve cell damage caused by frontotemporal dementia leads to loss of function in these brain regions, which variably cause deterioration in behavior, personality and/or difficulty with producing or comprehending language.
HIV-associated dementia	Occurs when the HIV virus spreads to the brain. Symptoms of HIV-associated dementia include loss of memory, difficulty thinking, concentrating, and or speaking clearly, lack of interest in activities and gradual loss of motor skills.
Parkinson's disease dementia	Is a decline in thinking and reasoning that develops in many people living with Parkinson's at least a year after diagnosis. The brain changes caused by Parkinson's disease begin in a region that plays a key role in movement, leading to early symptoms that include tremors and shakiness, muscle stiffness, a shuffling step, stooped posture, difficulty initiating movement and lack of facial expression.
Posterior cortical atrophy (PCA)	Refers to gradual and progressive degeneration of the outer layer of the brain (the cortex) in the part of the brain located in the back of the head (posterior).
Mixed dementia	Found in those who experience the brain changes of multiple types of dementia simultaneously.

2.8.4 Risk factors for dementia

Risk factors include age, genetics, lifestyle choices and health conditions

Table 9:

Modifiable risk factors	**Non-modifiable risk factors**
Low educational attainment Midlife hypertension	Age Genetic factors

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Midlife obesity Hearing loss Late-life depression Diabetes Physical inactivity Smoking Social isolation Alcohol abuse	

2.8.5 Causes of dementia

Dementia is caused by damage to or loss of nerve cells and their connections in the brain. Depending on the area of the brain that's affected by the damage, dementia can affect people differently and cause different symptoms. There can be reversible or non-reversible causes.

Dementias are often grouped by what they have in common, such as the protein or proteins deposited in the brain or the part of the brain that's affected. Some diseases present like dementias, such as those caused by a reaction to medication or vitamin deficiencies, and they might improve with treatment.

2.8.6 Signs and symptoms of dementia

Table 11: Common signs and symptoms of dementia:

Cognitive changes	Psychological and behavioral changes
• Problems with short-term memory loss, which is usually noticed by a spouse or someone else • Difficulty communicating or finding words • Difficulty with visual and spatial abilities, such as getting lost while driving	• Personality changes • Depression • Anxiety • Inappropriate behavior • Paranoia (a thought process that causes you to have an irrational suspicion or mistrust of others) • Agitation • Hallucination (sensory

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• Difficulty reasoning or problem-solving • Difficulty handling complex tasks • Difficulty with planning and organizing • Difficulty with coordination and motor functions • Confusion and disorientation	experiences that appear real but are created by one's mind) • Getting lost (wandering)

2.8.7 Management of dementia

If dementia is suspected, individuals should be referred to a health facility to be evaluated and a diagnosis made. Both non-pharmacologic and pharmacologic interventions are needed to optimally manage the cognitive, behavioral and psychological symptoms of dementia.

Below we focus on the management process:

Table 12:

Management process	**Strategies**
Management goals	There is no cure for Alzheimer's disease, so the chief goals of treatment are to: • Maintain quality of life • Maximize function in daily activities • Enhance cognition, mood and behavior • Foster a safe environment • Promote social engagement, as appropriate • Plan for the future
Managing cognitive symptoms and functioning	• Provide regular orientation information to people with dementia to help them to remain oriented to time, place and person (e.g. day/date, weather, names of people) • Promote communication using materials such as newspapers, radio or TV programmes, family albums and household items

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	to promote communication, to orient them, to stimulate memories. • Use simple short sentences to make verbal communication clear, and minimizing competing noises, such as the radio, TV, or other people's conversation. • Keep things simple and avoid changes to routine

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Managing behavioral symptoms	-Behavioral and psychological symptoms of dementia (BPSD), especially agitation, aggression, depression and psychoses, are the leading causes for need for caregiver or institutionalisation. -Left untreated those symptoms can accelerate functional decline and reduce quality of life -Refer to the health facility for assessment of the presence of a treatable condition -Address the cause of the symptoms and it relation to the experience of the person living with dementia(PLWD) (television watching by other family members may cause agitation and anxiety to the PLWD) -Resolve or minimize challenges and obstacles to the patients' comfort, security and ease of mind Enhance communication Establish a routine ● Identify and refer for treatment any physical health problems affecting behaviour ● Consider environmental modifications, such as seating, safe wandering areas, signs ● Identify trigger events (e.g. shopping at a busy market) or factors (e.g. going out alone) leading to problem behaviours, and try to see if that can be modified ● Consider calming strategies, such as suggesting the person does activities they enjoy (e.g. going for a walk, listening to music, engaging in conversation)
Promotion of independence, functioning and mobility	● Plan for activities of daily living in a way that it maximizes independent activity, enhances function, helps to adapt and develop skills, and minimizes need for support.

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	• Link individuals to any available social supports in the community
Prevention	Refer to section on prevention
Monitoring of the condition	Refer for evaluation on a regular basis
Other therapies	• Music therapy, which involves listening to soothing music • Light exercise • Watching videos of family members • Aromatherapy, which uses fragrant plant oils • Massage therapy • Art therapy, which involves creating art, focusing on the process rather than the outcome
Importance of the caregiver	• Community support to reduce distress which is common among caregivers for people with dementia • Identify psychological and psychosocial impact on care givers. Assess the care givers' needs to ensure necessary support for their family and social life, work and health • Keep a calendar • Ask friends or other family members for help • Take care of their physical, emotional and spiritual health • Join a support group • Refer the care giver if they are suffering from psychological issues
Reduction of stigma	• Education involves providing factual information on stigmatizing conditions to replace inaccurate stereotypes and beliefs, and increasing affirming attitudes • Showcasing the achievements of people with dementia,

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	• Relationship-building, • Engaging in purposeful learning

2.8.8 Prevention of dementia

Unfortunately, there has not been shown a sure way of preventing dementia. However, there are steps that may be taken to delay the onset of dementia.

- **Keep the mind active.** Mentally stimulating activities, such as reading, solving puzzles and playing word games, and memory training might delay the onset of dementia and decrease its effects.
- **Be physically and socially active.** Physical activity and social interaction might delay the onset of dementia and reduce its symptoms. Move more and aim for 150 minutes of exercise a week.
- **Quit smoking.** Some studies have shown that smoking in middle age and beyond may increase one's risk of dementia and blood vessel (vascular) conditions. Quitting smoking might reduce this risk and will improve health.
- **Manage cardiovascular risk factors.** Treat high blood pressure, high cholesterol, diabetes, and overweight and obesity. High blood pressure might lead to a higher risk of some types of dementia.
- **Treat health conditions.** See the doctor for treatment if you experience hearing loss, depression or anxiety.
- **Maintain a healthy diet.** Eating a healthy diet is important for many reasons, a diet — rich in fruits, vegetables, whole grains, might promote health and lower the risk of developing dementia. Consumption of a healthy diet also improves cardiovascular health, which may help lower dementia risk.
- **Get quality sleep.** Practice good sleep hygiene, and talk to the health care provider if snoring loudly or experiencing periods where one stop breathing (sleep apnoea) or gasp during sleep.

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2.9 Psychoses

2.9.1 Definition of Psychoses

Psychoses is a mental disorder characterized by a disconnection from reality. Psychoses may occur as a result of a psychiatric illness such as schizophrenia. In other instances, it may be caused by a health condition, medication or Substance Use. People who are experiencing psychoses may have either hallucinations or delusions.

2.9.2 Delusions and hallucinations

Delusions and hallucinations are two very different symptoms that are both often experienced by people with psychoses. Delusions and hallucinations seem real to the person who is experiencing them.

2.9.3 Delusions

A delusion is a false belief or impression that is firmly held even though it's contradicted by reality and what is commonly considered true. There are delusions of paranoia, grandiose delusions, and somatic delusions.

People who are experiencing a delusion of paranoia might think that they are being followed when they aren't or that secret messages are being sent to them. Someone with a grandiose delusion will have an exaggerated sense of importance. Somatic delusion is when a person believes they have a terminal illness, but in reality, they're healthy.

2.9.4 Hallucinations

A hallucination is a sensory perception in the absence of outside stimuli. That means seeing, hearing, feeling, or smelling something that isn't present. A person who is hallucinating might see things that don't exist or hear people talking when they're alone.

2.9.5 Causes of psychoses

Causes of Psychoses are different, and the exact cause isn't always clear. There are certain illnesses that cause psychoses like:

- brain diseases such as Parkinson's disease, Huntington's disease, and some chromosomal disorders
- brain tumors or cysts

Some types of dementia may result in psychoses, such as that caused by:

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- Alzheimer's disease
- HIV, syphilis, and other infections that attack the brain
- some types of epilepsy
- stroke

2.9.5 Triggers of psychoses

There are also triggers like Substance Use, and other environmental factors. In addition, certain situations can lead to specific types of psychoses developing

2.9.6 Conditions that trigger psychotic episodes

The following conditions have been known to trigger psychotic episodes in some people:

- Schizophrenia – A mental health condition that causes hallucinations and delusions
- Bipolar disorder – a person with bipolar disorder can have episodes of low mood (depression) and highs or elated mood (mania)
- Severe stress or anxiety
- Severe depression – feelings of persistent sadness including postnatal depression, which some women experience after having a baby
- Lack of sleep

2.9.7 Drugs known to trigger psychotic episodes

The following drugs have been known to trigger psychoses:

- cocaine.
- amphetamine (speed)
- methamphetamine (crystal meth)
- mephedrone (MCAT or miaow)
- MDMA (ecstasy)
- cannabis.
- LSD (acid)
- Psilocybins (magic mushrooms)

2.9.10 Risk factors for developing psychoses includes;

- People are more likely to develop a psychotic disorder if they have a close family member, such as a parent or sibling, who has a psychotic disorder.

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- Children born with the genetic mutation known as 22q11.2 deletion syndrome are at risk for developing a psychotic disorder, especially schizophrenia.

2.9.11 Types of psychoses

1. Brief psychotic disorder

Brief psychotic disorder, sometimes called brief reactive psychoses, can occur during periods of extreme personal stress like the death of a family member. Someone experiencing brief reactive psychoses will generally recover in a few days to a few weeks, depending on the source of the stress.

Drug- or alcohol-related psychoses

Psychoses can be triggered by the use of alcohol or drugs, including stimulants such as methamphetamine and cocaine. Hallucinogenic drugs like LSD often cause users to see things that aren't really there, but this effect is temporary. Some prescription drugs like steroids and stimulants can also cause symptoms of psychoses. People who have an addition to alcohol or certain drugs can experience psychotic symptoms if they suddenly stop drinking or taking those drugs.

Organic psychoses

A head injury or an illness or infection that affects the brain can cause symptoms of psychoses. The following are major Psychiatric Disorders that may have psychotic symptoms:

2.9.12 Bipolar disorder

When someone has bipolar disorder, their moods swing from very high to very low. When their mood is high and positive, they may have symptoms of psychoses. They may feel extremely good and believe they have special powers.

When their mood is depressed, the individual may have psychotic symptoms that make them feel angry, sad, or frightened. These symptoms include thinking someone is trying to harm them.

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2.9.13 Delusional disorder

A person experiencing delusional disorder strongly believes in things that aren't real.

2.9.14 Psychotic depression

This is major depression with psychotic symptoms.

2.10 Schizophrenia

Schizophrenia is a lifelong disease that is generally accompanied by psychotic symptoms.

2.10.1 Diagnosis of psychoses

Psychoses is diagnosed through a psychiatric evaluation, e.g.; by conducting a Mental status examination. That means a doctor will watch the person's behavior and ask questions about what they're experiencing. Medical tests and X-rays may be used to determine whether there is an underlying illness causing the symptoms.

2.10.2 Management of psychoses

Treating psychoses may involve a combination of medications and therapy. Most people will experience an improvement in their symptoms with treatment.

2.10.3 Psychotherapy

2.10.3.1 Cognitive behavioral therapy

- Psycho-education by explaining what psychoses is to the person and their family. Monitor symptoms of psychoses through means such as keeping a daily log of treatment and behavior.
- Cognitive behavioral therapy means meeting regularly to talk with a mental health counselor with the goal of changing thinking and behaviors.
- This approach has been shown to be effective in helping people make permanent changes and better manage their illness.
- It's often most helpful for psychotic symptoms that don't completely resolve with medications.

2.10.3.2 Prevent Relapses

- Compliance with treatment and appointments
- Refer back to health facility if symptoms are observed (even prior to return date).

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- A person in the psychotic state might not want to seek care, therefore it is important for caregivers to be part of relapse prevention.
- Professional to build the trust of the person with psychotic disorders. This might also improve treatment adherence.
- Build Relations: It is important for the health
- Resume positive social activities, such as hobbies, exercise, social networks, community functions.

2.10.3.3 Community support

- Community education and advocacy for integration of persons living with schizophrenia
- Establish a community treatment support system (treatment buddy for drug adherence)
- Community income/ meaningful work support system
- Establish a support network to enhance a sense of connection

2.10.4 Prevention of Psychoses

1. Emphasize on the importance of sufficient rest and sleep
2. Emphasize on nutritious and diversified diet
3. Emphasize on some physical activity
4. Spend time outside
5. Avoid drugs and alcohol
6. Referral to seek early treatment
7. Follow up appointments
8. Adherence to drug regimen
9. Encourage support groups
10. Family support

2.10.5 Alcohol Use Disorders

These are conditions resulting from different patterns of alcohol consumption including acute alcohol intoxication, harmful alcohol use, the alcohol dependence syndrome, and the alcohol withdrawal state.

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2.10.6 Terms Associated with Alcohol use;

Acute intoxication: a transient condition following intake of alcohol resulting in disturbances of consciousness, cognition, perception, affect or behaviour.

Harmful use: a pattern of alcohol consumption that is causing damage to health. The damage may be physical (as in cases of infections related to drug use) or mental (e.g. episodes of depressive disorder) and is often associated with damage to social functioning (e.g. family problems, legal problems or work-related problems).

Alcohol dependence: a cluster of physiological, behavioral and cognitive phenomena in which the use of alcohol takes on a much higher priority for a given individual than other behaviors that once had greater value.

Alcohol withdrawal state: refers to a group of symptoms that may occur upon cessation/ reduction of alcohol after its prolonged daily use. Various factors can impact how drinking alcohol affects your body and behavior.

2.10.6.1 Factors Contributing to Alcohol Use Disorders

a. Genetic,
b. Psychological,
c. Social and
d. Environmental factors

Many varied factors are associated with one being involved with alcohol and or drug use. These include the following as shown in figure 1:

Micro-level Environmental Factors:

- Family
- School
- Peers.

Macro-level Environmental Factors:

- Poverty
- Social environment
- Physical environment.

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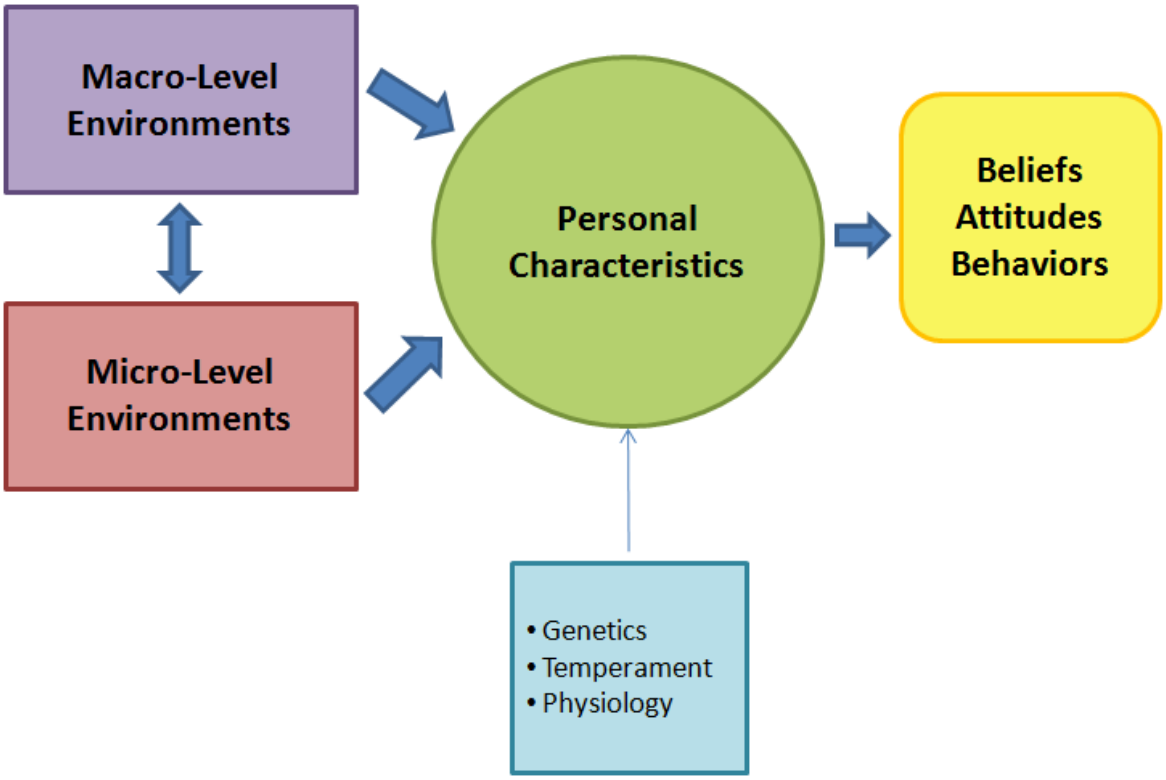

Figure 4: Factors Associated with Alcoholism

2.10.6.2 Signs and Symptoms of Alcohol Use Disorder

- Alcohol is taken in larger amounts or over a longer period than was intended.
- Persistent desire or unsuccessful efforts to cut down or control alcohol use.
- A lot of time is spent in activities necessary to obtain alcohol, use alcohol, or recover from its effects.
- Craving, or a strong desire or urge to use alcohol.
- Recurrent alcohol use resulting in a failure to fulfil major role obligations at work, school, or home.
- Continued alcohol use despite having persistent or recurrent social or interpersonal problems caused or exacerbated by the effects of alcohol.
- Important social, occupational, or recreational activities are given up or reduced because of alcohol use.
- Recurrent alcohol use in situations in which it is physically hazardous.
- Continued use of alcohol despite the knowledge of physical and psychological harm.
- A need for markedly increased amounts of alcohol to achieve intoxication or desired effect

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NOTE: The presence of at least 2 of these symptoms indicates an alcohol use disorder (AUD). The severity of an AUD is graded *mild*, *moderate*, or *severe***:**

2.10.6.3 Management of Alcohol Use Disorders

Behavioral Management

Behavioral management are aimed at changing drinking behavior through counseling.

Medications

Medications are currently guided by treatment protocols to help people stop or reduce their drinking and prevent relapse.

i. **Mutual-Support Groups**

Alcoholics Anonymous (AA) provide peer support for people quitting or cutting back on their drinking. Combined behavioral and medical management, can lead to better outcomes.

Life course approach to interventions on Alcohol use Prevention

2.10.6.4 Preventive Measures for Alcohol Use Disorders

Pregnancy, Infancy and Early Childhood

These are meant to help mothers understand the causative agents and how exposure to alcohol can impact the general health as well as the growth of the brain of the child. Interventions entail: sensitization on how to protect the unborn child from the effects of alcohol use; Sensitization on how to protect the growing baby from the negative effects of alcohol by engaging both the parents and the community strategies include;

- targeting pregnant women with alcohol use disorders
- Prenatal, Antenatal, Postnatal and infancy visitation Early childhood education
- Community-based multi-component

Middle Childhood

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These include interventions to help parents learn new skills on parenting to help them be able to bring up kids who are not predisposed to alcoholism. Examples are skills on how to handle pressure, disciplining the child, positive stress coping mechanisms, positive environment for the growing child etc. The interventions include;

- Parenting skills programmes
- Personal and social/life skills education
- Community based -multi-component (community stakeholder involvement)
- Classroom environment improvement programmes

Early Adolescence

These are interventions targeting parents and the community on how to handle adolescents who are beginning to develop abstract thinking and curiosity. Interventions include:

- Parenting skills programmes
- Prevention education - based on personal and social skills and social influences
- School policy and culture
- Addressing individual psychological vulnerabilities

Adolescence

These are interventions that target adolescents themselves. At this stage, the adolescents can think abstractly and make a judgement based on their own conviction, or peer influence. Therefore, interventions for them include:

- Mentoring
- Alcohol and tobacco policies
- Community multi-component initiatives (Community stakeholders involvement)
- Media Campaigns
- Brief Intervention
- Behaviour change communication

Late Adolescence and Adulthood

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The interventions target the entire population in a community focusing on key stakeholders (the police, the law makers, the workplace, the various communities, the community influencers/spokesmen, the administrative unit etc.) The interventions include;

- Prevention education based on personal and social skills and **social influences**
- School policy and culture
- Addressing individual psychological vulnerabilities
- Alcohol and tobacco policies
- Community multi-component initiatives
- Media Campaigns
- Workplace prevention
- Entertainment venues
- Brief intervention
- Behaviour change communication
- Mentorship and coaching

2.11 Drug Use Disorders

2.11.1 Definition of Terms Associated with Drug Use Disorder

Drug use disorder is a condition resulting from different patterns of drug use, including overdose, intoxication, harmful or hazardous drug use.

Due to their psychoactive and dependence-producing properties, psychoactive substances have the potential to harm individuals' health and lead to societal harm. The nature and severity of the harm relate to many factors, including the properties of the psychoactive substance itself, the form and mode of administration used, the characteristics of the person taking the substance, and the social context in which it is taken.

Terms associated with drug use disorders

1. Drug: is any substance other than food that affects the function of the body

2. Drug abuse

This is the misuse or overuse of a substance in a way different from the way it is generally used either in medical or social circumstances. The term also refers to the use of any illegal substance including alcohol if one is under-age as well as continued use of a substance even though it is causing problems in one's life.

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3. Acute intoxication: a transient condition following use of drugs resulting in disturbances of consciousness, cognition, perception, affect or behaviour.

4. Harmful use: a pattern of drug use that is causing damage to health. The damage may be physical (as in cases of infections related to drug use) or mental (e.g. episodes of depressive disorder) and is often associated with damage to social functioning (e.g. family problems, legal problems or work-related problems).

5. Drug dependence: a cluster of physiological, behavioral and cognitive phenomena in which the use of drugs takes on a much higher priority for a given individual than other behaviors that once had greater value.

6. Drug withdrawal state: refers to a group of symptoms that may occur upon cessation/ reduction of drugs after prolonged use

2.11.2 Causes of Drug Use Disorders

i. Genetic /Hereditary factors
ii. Environmental, noise, extremes of temperatures
iii. Availability of drug
iv. Curiosity
v. Social factors- Adverse life events/ -break ups, divorce, separation, bereavement, displacement
vi. War, conflict & violence
vii. Underlying medical and Psychiatric conditions
viii. Trauma, especially head injury
ix. Peer pressure
x. Sexual malfunction /dysfunction

2.11.3 Diagnostic Criteria for Drug Dependence

For one to be diagnosed with drug dependency they need to present with the following:

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Table 3:

Presentation	**Description**
Tolerance	Need for higher doses to achieve the same effect.
Withdrawal Symptoms	Symptoms that develop when an individual who is physically addicted to a drug abstains (intentionally or otherwise) and levels of the chemicals in the body begin to diminish or reduce
Impaired control	Persistent desire to use substance, unsuccessful attempts at controlling use, or using for longer period than intended
Neglecting activities	Alternative activities are neglected or abandoned due to substance use
Time spent using drugs	Much time is devoted to obtaining, using, or recovering from the effects of substance use

2.11.4 Classification of Drugs by Effect

Table 4: Classification of drugs by effects

Depressants	**Stimulants**	**Hallucinogens**	**Inhalants**
Alcohol	Amphetamines	LSD, DMT	Aerosols e,g, spray paint, hair and deodorant sprays, whipped cream dispensers, fabric protector sprays, and vegetable oil cooking sprays
Benzodiazepines	Methamphetamine	Mescaline	Gases e.g., butane (from lighters), propane (gas grills),

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			and cooling system fluids, medical anesthetic gases, such as ether, chloroform, halothane, and nitrous oxide
Opioids	Cocaine	PCP	Volatile Solvents e.g paint thinners and removers, dry cleaning fluids, degreasers, and gasoline glues, correction fluids, felt-tip marker fluids, and electronic contact cleaners
Solvents	Nicotine	Ketamine	Nitrites e,g, myl, butyl, and cyclohexyl nitrites
Barbiturates	Khat	Cannabis (high doses)	
Cannabis (low doses)	Caffeine	Magic mushrooms	
	MDMA	MDMA	

2.11.5 Classification of Drugs by Effect

Table 5: Drug Effects

Type of Drug	**Effects**
Depressants	Depressants are drugs that impair the activity of the central nervous system by enhancing the effects of GABA, a neurotransmitter which inhibits connectivity among neurons. **DEPRESSANTS**: SHORT-TERM EFFECTS

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	• Slow brain function • Slowed pulse and breathing • Lowered blood pressure • Poor concentration • Confusion • Fatigue2 • Dizziness • Slurred speech • Fever • Sluggishness • Visual disturbances • Dilated pupils • Disorientation, lack of coordination • Depression • Difficulty or inability to urinate • Addiction Long-term use of depressants can produce: • depression, • chronic fatigue, • breathing difficulties, • sexual problems and • Sleep problems. As a dependency on the drug increases, cravings, anxiety or panic are common if the user is unable to get more. Withdrawal symptoms include insomnia, weakness and nausea. For continual and high-dose users, agitation, high body temperature, delirium, hallucinations and convulsions can occur. Unlike withdrawal from most drugs, withdrawal from depressants can be life threatening.
Stimulants	Stimulants are a class of drugs, which enhance central nervous system activity and block

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	the re-uptake of the neurotransmitters norepinephrine and dopamine. The short-term effects of stimulants can include: • Euphoria • Accelerated alertness • Enhanced energy • Increased arousal • Rapid heart rate • Unusual behavior The extended use of stimulants can cause severe effects on the body such as: • Tremors • Tachycardia • Insomnia • Tooth decay • Heart attack • Brain damage • Organ damage • Sudden death
Hallucinogens	Hallucinogens are mind-altering drugs which cause false perceptions of reality. The short-term effects of Hallucinogens can include: • Hallucinations • Fluctuations in heart rate • Intense feelings and sensory feedback • Feelings of distorted time • Dry mouth • Anxiety or paranoia The extended abuse of some Hallucinogens may produce a variety of long-term effects, including: • Speech problems and memory loss from repeated PCP use • Bladder ulcers and kidney problems from Ketamine use • Paranoia

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	• Visual disturbances • Mental problems • Flashbacks
Inhalants	Inhalants are volatile substances that produce chemical vapors that can be inhaled to induce a psychoactive, or mind-altering, effect. Although other abused substances can be inhaled, the term "inhalants" is used to describe a variety of substances whose main common characteristic is that they are rarely, if ever, taken by any route other than inhalation. Some of the more common negative effects of inhalant use include: • Brain damage • Liver damage • Kidney failure • Respiratory distress • Blindness • Intestinal inflammation • Ulcers • Hallucinations • Cardiac arrest • Loss of sense of smell

2.11.6 Management of drug use disorders

Entails multiple approaches that include:

I. **Detoxification:** As one stops taking drugs, allowing the drugs to leave the body. One may need healthcare supervision to detox safely.

II. **Medication-assisted therapies:** During detox, medicine can help control cravings and relieve withdrawal symptoms.

III. Psychosocial Support:

- **Behavioral therapies:** Cognitive behavioral therapy or other psychotherapy (talk therapy) can help deal with addiction's cause. Therapy also helps build self-esteem and teaches healthy coping mechanisms.

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2.11.7 Prevention of substance use disorder

The focus in prevention of substance use is to help people, particularly young people,

- Avoid or delay the initiation of the use of substance use,
- If they have already started using substances, link to relevant services to prevent the development of dependency.
- School-based awareness programmes for children to impart the messages to them at an early age so that they can make healthy choices in life when they grow up.
- Youth groups in the community, who are important role models, may be roped in through awareness programmes and making use of them to in turn to spread the message.
- Women self-help groups can be educated as members of such groups may be affected by the family member's drug use and most importantly become co-dependent.
- Local politicians/ influencers need to be addressed, as they can support the cause.
- Religious heads from the community need to be involved, as they will positively influence the community to seek professional help.
- County governments can be periodically approached to give out appropriate public service messages.
- Local NGOs working in the community need to be made as the focal point of contact.

Awareness needs to be created about the help that is available for the family, educational institution, substance users. This will call for:

- Creating awareness of the different toll-free numbers for affected individuals or their families or friends to approach.
- Disseminate Posters/pamphlets/notices to be displayed.
- Seeking help from the media including print, TV and radio (community media)
- The community health volunteers should be trained on referral and follow up.
- Primary care givers need to be sensitized on substance use disorders prevention
- Community dialogue days can be conducted in partnership with appropriate treatment centres and a host organization from the community.
- Observing all important days like International day against illicit drugs and trafficking, Anti-tobacco day, etc., helps to increase awareness about the problem of substance abuse and addiction.

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- Law/police departments need to be engaged to prevent legal problems associated with substance use and intervene for persons with substance use in conflict with the law.
- Use of social media is an increasingly powerful tool even in rural areas. This can work in both enhancing awareness on treatment as well as be exploited by substance using networks.

NB: Prevention of substance use disorders in communities calls for a multi-pronged approach and must address the various stages of the life cycle of the population and various segments of the community.

UNIT 3: DRUGS AND SUBSTANCE ABUSE

DEFINITIONS OF TERMS

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A substance is considered abused if it is deliberately used to induce physiological or psychological effects or both, for the purposes other than therapeutic ones, and when its use contributes to health risks, the disruption of psychological functioning, adverse social consequences or a combination of them.

A drug can be defined as a substance, which may be chemical, synthetic or natural, which when taken in by a living organism may modify or affect one or more of its functions

Drug misuse is considered to be the misuse of medicine and incorrect use of drugs or medicines with the intention of causing body changes. It can also be said to be taking of medicine/drugs without the doctor's prescription

Commonly Abused Drugs and Substances

The most commonly abused substances are alcohol and tobacco. They are in fact termed as gateway substances to other drugs.

Alcohol

Alcohol includes wine, spirits, beer, “busaa”, “muratina”, “mnazi”, “changaa" and others such as “kumi kumi” “machozi” etc). Alcohol is considered a depressant which slows and lowers the functions of the brain, e.g. thinking, concentration, and recognition, making decisions and initiating reasoned actions. The effect of alcohol on the brain make people feel relaxed, stop worrying about what other people think of them, and have a good time.

The initial effect of alcohol makes one feel stimulated, become talkative and more active. As some continue taking more alcohol, mood and social behavior changes, some people become depressed and remorseful; others become belligerent (become abusive orviolent)

Depressive drunks may slow down, stumble, loudly confess their sins and failures and slur in words. The Amoral drunks may pick up quarrels and fights and other reckless behaviors. When they fall asleep they wake up with a hangover which includes fatigue, headache and nausea. They may also feel restlessness during the day and this can affect their productivity

Tobacco

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Tobacco contains 4,000 different chemicals many of which are harmful. Nicotine, carbon monoxide and tar are the three main chemicals that affect the human body and cause diseases. Tobacco can be smoked, snorted or chewed

Effects of Tobacco

The tar in the cigarette is deposited in the lungs causing lung damage and even lung cancer. Carbon monoxide present in cigarettes also affects the chemical activity of the heart, which encourages deposits on the walls of the arteries leading to blockage that may cause blood circulation problems.

For pregnant women this reduces the amount of oxygen reaching the baby during pregnancy. Smoking can also cause spontaneous abortion and other complications during pregnancy, low birth weight babies and stillbirths as well as prenatal mortality in women

Smoking also leads to increased incidences of severe coughing, sneezing and shortness of breath on exertion. Peptic ulcers in the stomach and duodenum, cancer of the mouth, nose, throat, esophagus, pancreas, bladder, cervix and leukemia are other diseases caused by smoking

Economically, although smoking contributes to a country's revenue, it eats into the family budget. It is also very costly in the treatment of diseases associated with smoking

Cannabis Sativa

It is also known as Bang, Marijuana (street names -grass, weed, pot, jive, reeter, and ganja. This drug is rolled into cigarette and smoked; and can also be chewed. It can also be processed into more potent form known as hashish.

Effects of Marijuana

- Problems with memory and learning
- Trouble in thinking and problem solving
- Loss of coordination
- Increased heartbeat, anxiety and panic attacks
- Some people can make stupid mistakes at work place
- It leads to unsafe sex and spread of HIV&AIDS
- It can lead to cancer, respiratory problems, immune system and reproductive system problems

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- It may lead to use of other illicit drugs
- It may lead to confusion and psychotic behavior

Glue

Health Risks

- Nausea, vomiting, blackouts and heart problems that can be fatal
- Squirting gas products down the throat may cause the body to produce fluid that floods the lungs and this can cause instant death
- Risk of suffocation if the substances inhaled from a plastic bag over the head
- Accidents can happen when the user is high because their senses are affected
- Long term abuse of glue can damage the brain, liver, and kidneys

Effect of Glue

- Users feel thick-headed, dizzy, giggly, and dreamy
- Users may hallucinate
- Effects do not last very long. But users can remain intoxicated all day by repeating the dose

Khat (Miraa)-is a stimulant

CAUSES OF DRUG AND SUBSTANCE ABUSE

- ✓ The following are some of the common causes of drug and substance abuse
- ✓ Pressure to perform well in examination
- ✓ Work/academic overload
- ✓ Family problems like rejection and a shaky family background
- ✓ Poverty
- ✓ Peer pressure
- ✓ Curiosity
- ✓ To escape from problem at home or workplace
- ✓ Boredom
- ✓ Poor parenting in terms of modeling as well as laxity among the parents hence loosing influence over their children's lives.
- ✓ Media influence

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- ✓ Social economic conditions and background, for example coming from "miraa"growing area
- ✓ Failure to develop prosocial values in early life
- ✓ Religious and cultural influences e.g. Rastafarianism.
- ✓ Desire to have another sense of holiness
- ✓ Leamers with excess pocket money may be tempted to abuse drugs

Signs and Symptoms of Drug and Substance Abuse

Physical Signs

These include:

- ✓ Slurred speech, staggering walk due to lack of co-ordination body movement, blurred and double vision, bloodshot eyes, watery eyes and drowsiness, unusual rashes and sores around the mouth and nose
- ✓ Sudden change in eating habits such as loss of appetitive or eating too much Burnt fingers, bumt holes in clothes ans injection marks
- ✓ General poor health
- ✓ Dry mouth or constant licking of lips Persistent cough
- ✓ Emergence of gangsterism with same mannerisms, for example , hair cut Untidiness-becoming untidy or tidy all of a sudden.
- ✓ Gathering in groups in isolated places or dejected buildings

Psychosocial signs

These signs include:

- ✓ Withdraw from the family members and friends. Being involved with other company and that is of suspicious behavior
- ✓ Mood swings. Victims may sometimes become violent, excessively excited or argumentative
- ✓ Confused conversation, impaired judgment and lack of proper concentration given tasks
- ✓ Behavior change where some may present extreme changes in behavior and personality e.g. being untidy or very neat all of a sudden
- ✓ Others may present blank, dreamily expressions, fear of lack of sleep Truancy and refusal to go to school

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- ✓ Despair and loss of interest in life, which may lead to depression
- ✓ Mental sickness may set in. Victims experience hallucinations and delirium such as seeing things that are not real.
- ✓ Drugs could also lead to suicide Compulsive drug seeking

EFFECTS OF DRUG AND SUBSTANCE ABUSE

A drug abuser undergoes several stages

Stage one: Tolerance

A person starts abusing drugs for the first time and more into habitual use where the drug takes the central stage of his/her life. This repeated use leads to changes in the brain and nervous system so that the user needs more of the drug to get the expected effect

Stage Two: Dependence

The body of the drug abuser demands that he takes more drugs. The abuser is now in the stage of psychic or physical compulsive desire to continually use the drug, either to experience the desired effects or avoid the consequences of withdrawal. The suffering from withdrawal is like an emotional prison trapping the drug abuser.

Stage Three: Addiction

The user is now a victim of drugs and cannot help himself or herself. He/she has now developed physical and psychological need for the drug for the body to function normally. This may lead to damage of vital organs in the body such as lungs, liver, central nervous system etc. and the work output may deteriorate

The user lives in denial and has little concern for future life. The use may manipulateother people to get what he/she wants so as to maintain the habit.

RELATIONSHIP BETWEEN SUBSTANCE ABUSE AND HIV & AIDS

Sharing of needles by substance abusers can lead to HIV infection and AIDS

Substance abusers stand a high risk of spreading HIV &AIDS through sexual intercourse

Substance use is known to impair a person's judgment and for this reason the person may expose himself/herself to the virus

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The uncontrollable properties of psychoactive substances lead to sexual and other highrisk behaviors that individuals might otherwise avoid

MANAGEMENT OF DRUG AND SUBSTANCE ABUSE

Detoxification Process

This is getting the drug out of the person's system and seeing him/her through the withdraw system, where drugs dependency starts

Counseling

Characteristics of clients who are motivated to change

·Accept the diagnosis given

Uncomfortable about the behavior

· Talk about change willingly

· Agree with the counselor

Follow the course of action

·They will start, continue and try to sustain change

NB:The counselor's role is to motivate the client and affirm any small achievement

Effective Motivation Strategy

Motivation is a state of readiness or eagerness to change, which may fluctuate from one time situation to another. This stage is one that can be influenced. Prochaska & Diclemente developed a wheel of change model that shows the stages through which clients pass in course of changing a problem. These include:

i. Pre-contemplation

1. A person is not considering change, has not contemplated having a problem or needing to make a change. People are very defensive here

2. People seldom present themselves for treatment but they are made under coercion

3.They need information and feedback to raise awareness about the problem and the possibility of change

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ii. Contemplation

1. Awareness is already raised but the client considers change and rejects it. They are ambivalent

2.When they talk about self, they go back and forth between reasons for concern and justification for un concem (caused by pathological personality traits or defense mechanisms)

3. The counselor task at this stage is to help tip the balance in favor for change

4. People come for consultation in the contemplation stage. As a counselor practice motivational interviewing

iii. Determination Stage

This is a window of opportunity. The counselor helps the client to match i.e. helping client to find a change strategy that is acceptable, accessible, appropriate and effective

iv. Action Stage

1. This is what most people think is therapy

2. The person engages in particular actions intended to bring about change

3. These changes may or may not be assisted by formal counseling. the goal during this stage is to produce a change in the area

v. Maintenance Stage

1. the challenge is to sustain the change accomplished by previous action to prevent relapse

2. maintaining change may require a different set of skills and strategies e.g. quitting a drug, reducing drinking or loosing weight is an initial step followed by the challenge of maintaining assistance or moderation

vi. Relapse

Help the client to renew the process of contemplation, determination and action stages without becoming stuck or demoralized because of the relapse

Motivational Interviewing

What is motivational interviewing?

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Motivation is defined as a probability that a person will enter into, continue and adhere to specific change strategy. It is the counselor's responsibility not only to dispense advice but also to motivate - to increase likelihood that the client will follow a recommended course of action to change. A counselor is like a salesman and motivation is inherent and central part of the professional task.

Principles of Motivation Interviewing

Some people need this, once unstuck, no longer immobilized by conflicting motivations, they have the skill and resources they need in order to make a lasting change

Important to people who are reluctant to change and endure ambivalent in changing

For others, motivational interviewing is a prelude to treatment

It creates openness to change which paves way for further therapeutic work Responsibility for change relies on the individual

This does not denote that therapists are powerless or helpless, they exert a lot of influence

· The strategies are more persuasive than coercive, more supportive than argumentative (the therapist creates a conducive environment for change)

The therapist is increasing client's intrinsic motivation, so that change arises from within rather than being imposed from without

Done properly, client presents the argument for change, rather than therapist.

Motivation interviewing employs a variety of strategies especially derived from person centered therapy

Counselor may appear relatively active

But the motivational interviewing proceeds with a strong sense of purpose, clear strategies and skills for pursuing that purpose and a sense of timing to intervene in particular ways at particular moments.

Motivational Interviewing Approach

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Systematically directs the client towards motivation for change Offers the counselor's own advice and feedback where appropriate Empathic reflection is used selectively to reinforce certain processes

Seeks to create and amplify the client's discrepancy in order to enhance motivation for change

De-emphasis on basis of labels; acceptance of “alcoholism” or other labels seen an unnecessary for change

Emphasize on personal choice and responsibility for deciding future behavior Therapists conduct the objective evaluation, but focuses on eliciting the client's own concems

Resistance is seen as interpersonal behavior pattem influenced by therapists behavior

Resistance is met with reflection

Treatment goals and change strategies are negotiated between the client the therapist, based on data and acceptability. Client's involvement in the process and acceptance of goals are vital

Treatment Approaches

1. Individual approaches

2. In patient counseling

3. Participation in AA (Alcohol Anonymous) support groups

4. Change can be enhanced by Cognitive approaches, Behavioral modification, Person centered Therapy, Solution Focused Brief Therapy

Re-integration

·Counseling dysfunctional families and children - they suffer from shame, anger,distorted thinking

The clients need to appreciated and accepted by the community to help them become productive members of the society

PREVENTIVE MEASURES TO DRUG AND SUBSTANCE ABUSE

Proper education on factors that contribute to the problem and deal with them There is need to educate the public on proper use of legal drugs

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Create awareness in the community, at work place, in schools and places of worship. Educating people about drugs removes ignorance

Media campaigns to highlight negative effects of drug use

Effective parenting helps to raise a drug free child. Functional families nature a normal child.

Occupational therapist to train workers how to cope with stress at work place Teach life skills and other skills related to their jobs e.g. interpersonal skills, assertiveness etc.

To detect the drug users and, help them through drug counseling,treatment and rehabilitations

Effective govemment policies on drug trafficking

LIFE SKILLS NECESSARY IN THE PREVENTION OF DRUG AND SUBSTANCE ABUSE

·Assertiveness

·Self-awareness

·Self-esteem

·Communication

·Decision-making

VALUES NECESSARY IN THE PREVENTION OF DRUG AND SUBSTANCE

- ABUSE
- ·Integrity
- Love
- ·Freedom
- Responsibility

Phobic Disorders

Definition of Phobia

A phobia is an excessive and irrational fear reaction. Persons with phobia may experience a deep sense of dread or panic when they encounter the source of the fear.

Type of Phobias

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1. Agoraphobia

Agoraphobia is a fear of places or situations that you cannot escape. The word itself refers to "fear of open spaces." People with agoraphobia fear being in large crowds or trapped outside the home. They often avoid social situations altogether and stay inside their homes.

2. Social phobia

Social phobia is also referred to as social anxiety disorder. It refers to extreme worry about social situations and it can lead to self-isolation. A social phobia can be so severe that the simplest interactions, such as ordering at a restaurant or answering the telephone, can cause panic. People with social phobia often go out of their way to avoid public situations.

3. Glossophobia

This is known as performance anxiety, or the fear of speaking in front of an audience. People with this phobia have severe physical symptoms when they even think about being in front of a group of people. Glossophobia treatments can include either therapy or medication.

4. Acrophobia:

This is the fear of heights. People with this phobia avoid mountains, bridges, or the higher floors of buildings. Symptoms include vertigo, dizziness, sweating, and feeling as if they'll pass out or lose consciousness.

5. Claustrophobia

This is a fear of enclosed or tight spaces. Severe claustrophobia can be especially disabling if it prevents you from riding in cars or elevators.

6. Aviophobia:

This is also known as the fear of flying.

7. Dentophobia:

Dentophobia is a fear of the dentist or dental procedures. This phobia generally develops after an unpleasant experience at a dentist's office. It can be harmful if it prevents you from obtaining needed dental care.

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8.Hemophobia:

This is a phobia of blood or injury. A person with hemophobia may faint when they come in contact with their own blood or another person's blood.

9.Arachnophobia: This means fear of spiders.

10. Cynophobia: This is a fear of dogs.

11. Ophidiophobia: People with this phobia fear snakes.

12. Nyctophobia:

This phobia is a fear of the nighttime or darkness. It almost always begins as a typical childhood fear. When it progresses past adolescence, it is considered a phobia.

Risk factors of Phobic Disorders

- Genetic and environmental factors can cause phobias. Children who have a close relative with an anxiety disorder are at risk of developing a phobia.
- Distressing events, such as nearly drowning, can bring on a phobia.
- Exposure to confined spaces, extreme heights, and animal or insect bites can all be sources of phobias.
- Age
- Socioeconomic status
- Gender seem to be risk factors only for certain phobias. For example, women are more likely to have animal phobias.
- Children or people with a low socioeconomic status are more likely to have social phobias.

Symptoms Associated with Phobias

The most common and disabling symptom of a phobia is a panic attack. Features of a panic attack include:

- Pounding or racing heart
- Shortness of breath
- Rapid speech or inability to speak
- Dry mouth
- Upset stomach
- Nausea

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- Elevated blood pressure
- Trembling or shaking
- Chest pain or tightness
- A choking sensation
- Dizziness or lightheadedness
- Profuse sweating
- A sense of impending doom

A person with a phobia does not have to have panic attacks for accurate diagnosis, however.

Management of Phobic Disorders

Treatment for phobias can involve therapeutic techniques, medications, or a combination of both.

1. Cognitive behavioral therapy

Cognitive behavioral therapy (CBT) is the most commonly used therapeutic treatment for phobias. It involves exposure to the source of the fear in a controlled setting. This treatment can decondition people and reduce anxiety.

The therapy focuses on identifying and changing negative thoughts, dysfunctional beliefs, and negative reactions to the phobic situation. New CBT techniques use virtual reality technology to expose people to the sources of their phobias safely.

2. Medication

Antidepressants and anti-anxiety medications can help calm emotional and physical reactions to fear. Often, a combination of medication and professional therapy is the most helpful.

NOTE:

If you have a phobia, it is critical that you seek treatment. Overcoming phobias can be difficult, but there is hope. With the right treatment, you can learn to manage your fears and lead a productive, fulfilling life.

Prevention of Phobic Disorders

There are no measures to prevent phobic disorders but the following ways are ways to overcome:

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Relaxation – If you face a situation in which fear or panic are beginning to take hold, simple relaxation techniques can help. Relax your muscles and mind by forming images of a place you would rather be. This can release tension and bring you back down.

Face your phobia - avoiding fears only makes them scarier. Whatever your fear, if you face it, it should start to fade

Challenge it intellectually- If you begin to feel panic and anxiety when facing your fear, you need to challenge the thoughts you have about your anxiety

Breathe through panic- If you start to get a faster heartbeat or sweating palms, the best thing is not to fight it. Stay where you are and simply feel the panic without trying to distract yourself. Place the palm of your hand on your stomach and breathe slowly and deeply. The goal is to help the mind get used to coping with panic, which takes the fear of "fear" away.

Talk about it - Sharing fears takes away a lot of their scariness

Utilize the "AWARE" technique of facing panic attacks and allowing it to pass - The steps involved in "AWARE" are "Acknowledge and Accept," in which you accept the oncoming attack, realize that it's scary, but also accept that it is not dangerous. Once you have accomplished this, you need to "Wait and Watch." Here, you're not trying to run away from the fear, but are instead letting it wash over you. Next are the "Action" steps, in which you bring the panic under control with breathing and resume the fearful activity. Finally, you'll want to "Repeat" the steps if necessary, and then acknowledge that no matter how it feels now, the attack will come to an "End."

Play music games- during a panic attack muscles tense up. That's why its recommended that people practice muscle control by intentionally tightening their muscles for 10 to 15 seconds, and then relaxing them. This gives people a sense of empowerment, that they, and not their emotions, are in charge of their muscles,

Beware of the effects of caffeine- high caffeine intake or caffeine addiction can only heighten your anxiety and increase your reaction to phobias

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4.0 MYTHS AND MISCONCEPTION ABOUT MENTAL HEALTH

The Biggest Misconceptions About Mental Illness

Mental health has long been misunderstood.

Only until more recently, mental illnesses and mental health disorders were taboo topics. People were uncomfortable talking about these subjects or their mental health struggles. As a result, rumors and misunderstandings about mental illnesses ran wild. The media played no helping hand with exaggerated stories and extreme cases being showcased regularly. Although there has been progress, there are still several popular misconceptions about mental disorders that many people still believed to be true and may even be falsely spreading.

Common Mental Health Misconceptions & Myths

Although these common misconceptions about mental illness can seem innocuous, they can be damaging in several ways. These mental health myths continue to perpetuate toxic stereotypes that may keep someone from getting the help they need. Some may fear how others will view them if they come forward and ask for help. Others may avoid treatment because of the rumors about the type of care they will receive. This can leave many people struggling on their own and internalizing these negative views on mental illness that could ultimately make their problems worse.

Myth: Mental health problems don't affect me.

Fact: Mental health problems are actually very common. In 2020, about:

- One in five American adults experienced a mental health issue
- One in 6 young people experienced a major depressive episode
- One in 20 Americans lived with a serious mental illness, such as schizophrenia, bipolar disorder, or major depression

Myth: Children don't experience mental health problems.

Fact: Even very young children may show early warning signs of mental health concerns. These mental health problems are often clinically diagnosable, and can be a product of the interaction of biological, psychological, and social factors.

Half of all mental health disorders show first signs before a person turns 14 years old, and three-quarters of mental health disorders begin before age 24.

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Unfortunately, only half of children and adolescents with diagnosable mental health problems receive the treatment they need. Early mental health support can help a child before problems interfere with other developmental needs.

Myth: People with mental health problems are violent and unpredictable.

Fact: The vast majority of people with mental health problems are no more likely to be violent than anyone else. Most people with mental illness are not violent and only 3%–5% of violent acts can be attributed to individuals living with a serious mental illness. In fact, people with severe mental illnesses are over 10 times more likely to be victims of violent crime than the general population. You probably know someone with a mental health problem and don't even realize it, because many people with mental health problems are highly active and productive members of our communities.

Myth: People with mental health needs, even those who are managing their mental illness, cannot tolerate the stress of holding down a job.

Fact: People with mental health problems are just as productive as other employees. Employers who hire people with mental health problems report good attendance and punctuality as well as motivation, good work, and job tenure on par with or greater than other employees.

When employees with mental health problems receive effective treatment, it can result in:

- Lower total medical costs
- Increased productivity
- Lower absenteeism
- Decreased disability costs

Myth: Personality weakness or character flaws cause mental health problems. People with mental health problems can snap out of it if they try hard enough.

Fact: Mental health problems have nothing to do with being lazy or weak and many people need help to get better. Many factors contribute to mental health problems, including:

- Biological factors, such as genes, physical illness, injury, or brain chemistry
- Life experiences, such as trauma or a history of abuse
- Family history of mental health problems

People with mental health problems can get better and many recover completely.

Myth: There is no hope for people with mental health problems. Once a friend or family member develops mental health problems, he or she will never recover.

Fact: Studies show that people with mental health problems get better and many recover completely. Recovery refers to the process in which people are able to live, work, learn, and participate fully in their communities. There are more treatments, services, and community support systems than ever before, and they work.

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Myth: Therapy and self-help are a waste of time. Why bother when you can just take a pill?

Fact: Treatment for mental health problems varies depending on the individual and could include medication, therapy, or both. Many individuals work with a support system during the healing and recovery process.

Myth: I can't do anything for a person with a mental health problem.

Fact: Friends and loved ones can make a big difference. In 2020, only 20% of adults received any mental health treatment in the past year, which included 10% who received counseling or therapy from a professional. Friends and family can be important influences to help someone get the treatment and services they need by:

- Reaching out and letting them know you are available to help
- Helping them access mental health services
- Learning and sharing the facts about mental health, especially if you hear something that isn't true
- Treating them with respect, just as you would anyone else
- Refusing to define them by their diagnosis or using labels such as "crazy", instead use person-first language

Myth: Prevention doesn't work. It is impossible to prevent mental illnesses.

Fact: Prevention of mental, emotional, and behavioral disorders focuses on addressing known risk factors such as exposure to trauma that can affect the chances that children, youth, and young adults will develop mental health problems. Promoting the social-emotional well-being of children and youth leads to:

- Higher overall productivity
- Better educational outcomes
- Lower crime rates
- Stronger economies
- Lower health care costs
- Improved quality of life
- Increased lifespan
- Improved family life

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5.0 COMMUNITY MOBILIZATION AND AWARENESS RAISING:

Community mobilization is a way of engaging community members in all sectors of the community to promote individuals, families, and community actions and to identify community priorities, resources, needs and solutions to promoting good representation participation, good governance, accountability and peaceful changes. It involves bringing together stake holders to raise community awareness of the needs and demands for problem solving and delivery of resources, services and utilization. Strengthen community participation for sustainability and self reliance. It may involve door to door outreach, public meetings participatory activities.

It also involves on going and meaningful widespread participation to ensure involvement of key stakeholders with focus on the poor and the vulnerable groups including the mentally ill persons

- Steps to community awareness
- Determine the level of awareness of the community
- Design the awareness program based on responses from the community on their awareness levels.
- Have people or groups of people who will take correct messages to the community about what needs to be done in the community
- Find ways to become part and parcel of the community
- Show the community how much you value them

It does this by raising awareness

- Empowering individuals
- Empowering groups
- Creating enabling environment
- Sensitizing community on positive changes

Importance of community mobilization

- It empowers the community to identify and address their own needs and generating local solutions to problems
- Strengthen the ability of communities to work together towards a common goal
- Participation of community members in achievement of common goals
- It leads to sustainability of any given action

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- Help in development of adequate intervention
- Develop trust in activities
- Encouraging participation of all community members, the youth, families, communities, and many young groups.

DETECTING MENTAL HEALTH CONDITIONS:

1. Physical examination: your doctor will try to rule out physical problems that could cause your symptoms.
2. Lab tests: These may include, for example, a check of your thyroid function or a screening for alcohol and drugs
3. A psychological evaluation: A doctor or mental health professionals talks to you about your symptoms, thoughts, feeling and behavior patterns. You may be asked to fill out a questionnaire to help answer these questions.

SUPPORTING PEOLE WITH LONG-TERM MENTAL CONDITIONS:

1. Spiritual support.
2. Material support.
3. Physical support.
4. Stigma reduction.
5. Awareness creation
6. Policy development.
7. Discrimination reduction.
8. Medical support
9. Integration into community.
10. Mental health program creation.
11. Increased access to care.
12. Training
13. Capacity building.
14. Community empowerment.

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STIGMA AND DISCRIMINATION AND ABUSE OF PEOPLE WITH MENTAL ILLNESS:

Stigma is when someone sees you in a negative way because of your mental illness. Discrimination is when someone treats you in a negative way because of your mental illness.

Social stigma and discrimination can make mental health problems worse and stop a person from getting the help they need.

Avoid stigmatizing people with mental illness by seeing the person first and not labelling them with their mental illness.

You can help challenge stigma by speaking up when you hear people around you make negative or wrong comments about mental illness.

Stigma is when someone sees you in a negative way because of a particular characteristic or attribute (such as skin colour, cultural background, a disability or a mental illness). When someone treats you in a negative way because of your mental illness, this is discrimination.

Stigma happens when ap person defines someone by their illness rather than who they are as an individual. For example, they might be labelled' psychotic' rather than 'a person experiencing psychosis'.

For people with mental health issues, the social stigma and discrimination they experience can make their problems worse, making it harder to recover. It may cause the person to avoid getting the help they need because of the fear of being stigmatized.

THE HARMFULL EFFECTS OF STIGMA

Some of the effects of stigma include:

- Feeling of shame, hopelessness and isolation
- Reluctance to ask for help or to get treatment
- Lack of understanding by family, friend or others
- Fewer opportunities for employment or social interaction
- Bullying, physical violence or harassment
- Self-doubt-the belief that you will never overcome your illness or be able to achieve what you want in life.

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DEALING WITH STIGMA

Here are some ways you can deal with stigma:

Get the mental health treatment you need. Try not to let the fear of being labelled with a mental illness stop you from getting help.

Do not believe it. Sometimes if you hear or experience something often enough, you start to believe it yourself. Try not to let other people's ignorance influence the way you feel about yourself. Mental illness is not a sign of weakness and is rarely something you can deal with on your own. Talking about your mental health issues with healthcare professionals will help you on your road to recovery or management.

Do not hide away: many people with mental illness want to isolate themselves from the world. Reaching out to people you trust- family, friends, coaches, or religious leaders-can mean you get the support you need.

Connect with others. Joining a mental health support group –either online or in person-can help you deal with the feelings of isolation and make you realize that you are not alone in your feelings and experiences.

You are not your illness. Do not define yourself by your illness as other people might. Instead of saying 'I'm schizophrenic', say I have schizophrenia There is power in language.

It's not personal. Remember that other peoples' judgments often come from a lack of understanding rather than anything else. The judgments are made before they get to know you, so do not believe that their views have anything to do with you personally.

PROMOTION AND PREVENTION OF MENTAL ILLNESS.

Promotion is an approach aimed at strengthening positive aspects of mental health and psychosocial wellbeing, and is focused on empowering people to live healthy lives (e.g. by facilitating healthy lifestyles through policies, such as providing nutritious foods in school canteens or opportunities for physical exercise in accessible locations), rather than health being the sole domain of health professionals (National Research Council and Institute of Medicine, 2009). It includes – for example – components to foster pro-social behaviour, self-esteem, coping, decision-making capacity, but also universal interventions such as social and economic interventions to improve people's social determinants of health which would

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impact on their wellbeing. Prevention is an approach aimed at reducing the likelihood of future disorder in the general population or for people who are identified as being at risk of a disorder (Eaton, 2012; Tol *et al*., 2015). Prevention is further subdivided on the basis of the population targeted, into universal, selective and indicated (National Research Council and Institute of Medicine, 2009). Universal prevention, which includes strategies that can be offered to the whole population including individuals who are not at risk, based on the evidence that it is likely to provide some benefit to all (reduce the probability of disorder), clearly outweighs the costs and risks of negative consequences. Examples of common universal prevention interventions include the community-wide provision of information on positive coping methods (IASC, 2007) to help people feeling safe and hopeful, protection against human rights violations (e.g. gender-based violence), community-wide efforts to improve livelihoods as a key protective factor for mental health.

Selective prevention refers to strategies that are targeted to subpopulations identified as being at elevated biological, social or psychological risk for a disorder but who are asymptomatic or have very minimal symptoms. These interventions involve human, supportive and practical help covering both a social and a psychological dimension. They work through communication (asking about people needs and concerns; listening to people and helping them to feel calm), practical support (i.e. providing meals or water) and with a psychological approach including teaching stress management skills and helping people to cope with problems (World Health Organization, 2011); facilitation of community support for vulnerable individuals by activating social networks and communication; structured cultural and recreational activities supporting the development of resilience (National Research Council and Institute of Medicine, 2009), such as traditional dancing, art work, sports and puppetry. These activities may take place in equipped settings with the aim of increasing the children's sense of connectivity and safety (Tol *et al*., 2011).

Indicated prevention includes strategies that are targeted to individuals who are identified (or individually screened) as having detectable signs or symptoms which can foreshadow, precede and may sometimes – if left unaddressed – lead to a full diagnosable mental disorder based on an individual assessment. These interventions to prevent mental disorders may be delivered at individual or group level, in a variety of settings (antenatal and postnatal visits, home visits, community settings, schools, etc.). These interventions include psychosocial support for persons with subclinical levels of mental disorders (Purgato *et al*., 2019*a*), such as

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mentoring programmes aimed at children with behavioural problems; psychological first aid for people with heightened levels of psychological distress after exposure to severe stressors, loss or bereavement (Tol *et al*., 2015). This includes facilitator-guided self-help group interventions, as for example the WHO Self-Help Plus (Epping-Jordan *et al*., 2016; Purgato *et al*., 2019*a*).

ADULT MENTAL HEALTH.

Mental health problems and neurological disorders among older adults account for approximately 10 to 15 % of the total disability of the world elderly group 60 years and above

Older adults have higher chances of developing mental health problems due to substance abuse, diabetes, and other health conditions. As the world population increases does the number of the old with mental health conditions.

The most common mental and neurological disorders in the old are dementia, depression, anxiety, substance abuse. This conditions cases almost a half of the elderly deaths and self-harm.

It is important to consider the effect of mental health disorders in the elderly because this condition is always ignored, misdiagnosed and overlooked in this age group. Also the stigma surrounding the condition makes people reluctant to seek help

Risk factors to mental health among the elderly

- Life stressors
- Losses of capacity
- Decline in functional ability
- Reduced mobility
- Chronic pains
- Life experiences like bereavement
- Drop in social economic status
- Isolation
- Loneliness

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- Psychological depression

Older people are as well vulnerable to elderly abuse such as physical abuse, verbal abuse, psychological, financial, sexual, abandonment, neglect, loss of dignity and respect. These life experiences may lead the elderly to developing physical injury, psychological problems, depression and anxiety.

Treatment

It is important to prepare health providers with training and knowledge on management of age-related chronic diseases including mental neurological and substance abuse disorders

Development of suitable policies on long term and palliative care. development of age friendly services

Health promotion

This involves the following activities

- Promoting healthy aging
- Creating healthy conditions
- Creating environment that supports wellbeing of the elderly
- Provision of resources to meet the elderly needs
- Providing security and freedom good housing
- Social support
- Health and social programs
- Deal with elderly abuse
- Prompt recognition and treatment of mental disorders

NB: currently there is no known cure for dementia but much can be done to support and improve the lives of those with the condition which includes

Early diagnosis to promote early management

- Identify and treatment underlying conditions
- Detecting and managing challenging conditions
- Providing information and long term support to care.

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Good general health and social care is important for promoting older people's health, preventing disease and managing chronic illnesses. Training all providers in working with issues and disorders related to ageing is therefore important. effective community level primary mental health care for older is important

Supportive legislative environment based on accepted human rights standards is required to ensure the rights and highest quality of services to people with mental illness and their caregivers

CHILD MENTAL HEALTH

Mental health in children means reaching developmental and emotional milestones and learning healthy social skills and how to cope when there is problem

Mental health in children means positive quality of life and function at home, in school, and in the community

Mental disorder in children means changes in the ways children typically learn, behave or handle their emotions which causes stress and problem getting through the day.

Mental health in children is important to the overall health. mental disorder are chronic health conditions that last a long time and often don't go away completely without early diagnosis and treatment children with mental disorders can have problems at home, in schools and in forming friends and can continue to adulthood. Many children have fear and worries and may feel sad and hopeless from time to time. strong fear may appear at different times during development. persistent or extreme form of fear and sadness could be due to either anxiety or depression

Some common childhood mental disorder includes

Attention disorder
Anxiety disorder- fears
Depression – being sad
Behavior disorder
Learning and developmental disability
Autism
Substance use

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Signs and Symptoms

Symptoms of mental health disorder in children may start in early childhood or develop in teenage. most diagnosis is done during school age but some may pass without being diagnosed. Some symptoms may include the following,

Difficulties with plays
Difficulties in learning
Difficulties in speaking
Difficulties in controlling emotions

Prevention and control

It is important to know your child in order to help the child develop mentally

- Talk to the child
- Notice how the child behave at home, in school and with friends
- Notice how the child is able to control ether anger worries or sadness.
- Early diagnosis and treatment is equally important
- Teachers should identify children with problems and work with families and health professional to help children with mental health problems

Integration of people with mental health disorder in communities

Discrimination and abuse of persons with mental illness, children with mental health and developmental problems.

It is illegal for discrimination of the mentally ill in all situations including, work place, rejection, promotion, forced activity, denying job

If an individual is able to show that the impact of his mental condition may lead to disability and he is treated unfairly because of that condition then it illegal to do so. Mental health may lead to disability that is protected under the Equality Act. 2010. Some included are sex, age, race.

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- Discrimination can be direct
- Indirect
- Harassment
- Victimization
- Discrimination due to disability

References/Further Readings

1. Barlow, David H. (1998) Abnormal Psychology: an integrated approach. Brooks Cole

2. Halgin, R. (2003). Abnormal psychology: clinical perspectives on psychological disorder. Mc-Graw Hill

3. GoK – MoH (2020): Mental health training manual for Community Health Assistants.

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